MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 1 OF 28

MFT 703

Human Development Across the Lifespan

15 teaching weeks · 7 practicals · 6 worked cases · 1 figure

SEMESTER ONE

1.71 units · Prerequisite: none

> “You cannot assess a family without knowing what is developmentally ordinary.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 703 — Human Development Across the Lifespan

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 15 teaching weeks · Prerequisite: none

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 703	**Human Development Across the Lifespan**	One	1.71

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Course leader's summary. You cannot assess a family without knowing what is developmentally ordinary. This course covers physical, cognitive, language, social-emotional and moral development from conception to later life, always in family and cultural context — and it trains you to distinguish normal variation, delay, regression and symptom.

How to use this book

Development is not a list of ages; it is a set of **tasks** and a set of **negotiations** between a person and their family. Each week here gives you the developmental task, the family task that accompanies it, what goes wrong, and what it looks like in a consulting room. Use the milestone grid (Figure 23) as a reference you return to, not as a checklist to memorise.

1. Course description

Human development is the backbone of family assessment. A two-year-old's tantrum, a nine-year-old's lying, and a fifteen-year-old's secrecy are not three versions of "defiance": they are three different developmental moments that require three different responses. Clinicians who do not know this either pathologise normal development or miss genuine delay.

The course integrates **neurodevelopmental, cognitive, attachment, systemic and cultural** perspectives. It treats development as bidirectional: children shape parents as much as parents shape children, and families develop too (a theme developed in MFT 719).

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe physical, cognitive, language and social-emotional milestones from 0–18 and in adulthood	Exam
LO2	Distinguish normal variation, delay, regression and symptom in a presenting concern	Case assignment
LO3	Apply attachment theory to family assessment	Case assignment, exam
LO4	Explain developmental tasks of adulthood and later life and their family implications	Exam
LO5	Evaluate developmental theory critically, including its cultural assumptions	Essay
LO6	Communicate developmental information to parents without jargon or alarm	Skills assessment

3. Assessment

Component	Weight	Due
Developmental case analysis (2,000 words)	30%	Week 8
Parent-feedback role-play (10 min, observed)	20%	Week 11
Written examination	50%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Why developmental knowledge is clinical knowledge	Milestone self-audit
2	Prenatal development, temperament and the transition to parenthood	Temperament interview
3	Infancy: attachment, synchrony, regulation	Still-face observation
4	Toddlerhood: autonomy, language, limit-setting	Tantrum consultation role-play
5	Early childhood: play, theory of mind, initiative	Play observation
6	Middle childhood: industry, peers, school, self-concept	School-refusal case
7	Adolescence I: puberty, identity, autonomy	Adolescent interview
8	Adolescence II: risk, peers, digital life, mental health	Risk/development differential
9	Adult development: intimacy, work, generativity	Life-structure interview
10	Couple development across the lifespan	Couple timeline exercise
11	Parenting across stages; communicating with parents	**Observed parent feedback**
12	Later life: cognition, loss, caregiving, legacy	Caregiver family meeting
13	Culture, poverty and development	Critical reading seminar
14	When development goes wrong: delay, disorder, trauma	Differential diagnosis workshop
15	Integration: the developmental lens in assessment	Case presentations

Week 1 — Why developmental knowledge is clinical knowledge

Three clinical errors are prevented by developmental literacy:

1. **Pathologising the ordinary.** A two-year-old who says "no" to everything is doing the developmental task of autonomy, not manipulating the family.
2. **Missing the abnormal.** A five-year-old who has lost previously acquired language is not "being difficult"; regression is a red flag requiring urgent referral.

3. **Mis-timing the intervention.** A parent is told to "reason with" a three-year-old whose prefrontal circuitry cannot yet support the reasoning demanded. The intervention fails and the parent concludes they are failing.

Four categories you must distinguish — this distinction recurs in every case assignment:

Category	Definition	Example
Normal variation	Within the expected range, differing in style and pace	Walking at 11 months vs 15 months
Delay	Functioning below the expected range but progressing	No words at 24 months, but progressing
Regression	Loss of previously acquired skill	A four-year-old stops speaking
Symptom	Behaviour that signals distress rather than development	New soiling at seven after a bereavement



Figure 1.1 — Developmental milestones from 0 to 18: a screening map, not a diagnosis

PRACTICAL

Write your own developmental timeline from memory: first word, first step, a significant illness, a move, a loss. Then ask a parent or sibling to correct it. Note which events you had forgotten and who remembered them.

1.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	The three errors	Three cases in which developmental ignorance produced harm	Name the error in each case
10–25	Development as negotiation	The person's task and the family's task, side by side	Write the pair of tasks for one age
25–40	The four categories	Normal variation, delay, regression, symptom, tested on video clips	Classify eight clips
40–55	The screening map	The milestone grid, used as a reference rather than a checklist	Locate a real child on the grid
55–70	What the grid cannot tell you	Culture, opportunity, illness, trauma, family organisation	List five things a milestone cannot show
70–82	Talking to parents	The sentence that informs without alarming	Rewrite three alarming sentences
82–90	Why this course is a clinical course	The assessment, the case analysis, the exam	Choose a family for the case analysis

The teaching decision that makes the week work. The week must establish a habit of mind, not a body of facts: before any behaviour is judged, ask what is the developmental task here, and whose task is it? The lecture should therefore work from three video clips of the same behaviour at different ages — a child refusing to share at two, at six and at thirteen — and let the class discover that the identical act means three different things and requires three different responses. The facts about milestones can be read; the habit must be demonstrated.

The sentence the lecture must land. "Behaviour is only meaningful in relation to a developmental task." A student who can look at a nine-year-old's lying and ask what task the lying is serving — rather than whether the child is honest — has begun to think developmentally, which is the whole of this course's foundation and the first line of every case analysis they will write for the next fifteen weeks.

1.2 Core concepts

Development as a set of tasks. Each stage of life presents a person with a task — to become attached, to become autonomous, to become competent, to become oneself, to become a partner, to become a parent, to become old — and each task requires a negotiation with the family. The task language comes from Erikson, and this course uses it because it is clinically usable: it converts a behaviour into a question about what the person is trying to achieve, and a question about what the family is being asked to allow.

Development as bidirectional. The child shapes the parent as surely as the parent shapes the child. A temperamentally reactive infant produces a more cautious mother; a mother's depression produces a more withdrawn infant; and the two effects are not separable, which is why the clinical unit is the relationship rather than the individual. This

is the course's central systemic claim, and it is the reason the course belongs in a family therapy programme rather than in a psychology department.

Development as a negotiation between tasks. At every stage there are two tasks: the person's and the family's. The toddler's task is autonomy; the family's task is to allow autonomy while keeping the child safe. The adolescent's task is identity; the family's task is to tolerate the adolescent's separateness without abandoning them. Most presenting problems in family therapy are a stalled negotiation between the two, and the therapist's developmental question is: which task is stalled, and whose?

The four categories. Normal variation (within range, differing in style and pace); delay (below range, but progressing); regression (loss of an acquired skill); and symptom (behaviour that signals distress rather than development). The categories are clinical instruments, and their misuse produces the three errors listed above. The distinction between delay and regression is the most urgent: delay usually allows assessment over weeks, and regression demands referral within days.

Normative and non-normative events. Normative events are those expected at a stage — starting school, puberty, leaving home, retirement. Non-normative events are unexpected — a parent's death, a move, an illness, a war, a divorce. The family life cycle literature (MFT 719) describes the normative; the clinician's task is to assess how a family's non-normative events have intersected with the normative ones, because that intersection is where most symptoms live.

The developmental history as a clinical instrument. A developmental history is not a form; it is the most efficient way to locate a family's difficulty in time. Not "was he a difficult baby?" but "when did this start, what else was happening then, and what changed at that point?" The history converts a presenting complaint into a chronology, and the chronology usually shows that the symptom began at a transition — which tells the therapist where to look.

Screening is not diagnosis. A milestone grid identifies children who need a fuller assessment. It does not diagnose, and it cannot distinguish a child who is late because of a global delay from one who is late because of deprivation, illness, trauma or a family in crisis. A clinician who treats the grid as a diagnostic instrument will refer children who are developing normally and miss children whose development is being distorted by their circumstances.

Culture, opportunity and the milestone. Milestones describe what children do when they are given the conditions to do it. A child who has not learned to count by six may be delayed, or may never have been taught; a child who does not speak in the clinic may be shy, or may speak a different language at home, or may have been told not to speak to strangers. The clinical question is always: what has this child had the opportunity to learn, from whom, in what language?

Risk and resilience. The literature's most useful finding for family therapists: the number of risk factors matters less than the balance between risk and protective factors, and the single most powerful protective factor is a relationship with at least one

consistently responsive adult. That finding is the developmental justification for family therapy, and it should be quoted in every assessment that identifies a child at risk.

1.2a The developmental history, asked well

The instrument the whole course depends on, and the one most often performed as a form. Eleven questions, each with what it is for.

One — "start at the beginning: was this a planned pregnancy, and how was it for you?" For: the parents' state at the child's beginning, which predicts a good deal about the relationship's foundation. An unplanned pregnancy in a hostile household is not a verdict; it is information.

Two — "how was the birth, and how were the first weeks?" For: birth trauma, prematurity, a mother's illness, a period of separation. The question also signals that the therapist is interested in the parents' experience, not only the child's.

Three — "who was at home in the first year, and who helped?" For: the caregiving network, which in many families is wider than the parents, and which is often invisible in the referral.

Four — "when did he first smile, sit, walk, and say a word?" For: the developmental trajectory, asked in ordinary language rather than as a checklist. Parents remember these with surprising accuracy, and their confidence in answering is itself informative.

Five — "was he different from your other children, and in what way?" For: the comparison parents make spontaneously, and the meaning they have given it.

Six — "when did you first become worried, and what made you worried then?" For: the starting point of the parents' concern, which is frequently earlier than the referral, and the event that converted a characteristic into a problem.

Seven — "what was happening in the family at that time?" For: the intersection of the child's development with the family's non-normative events. This is the question that produces the chronology's turning points.

Eight — "who has said what about it — school, clinic, family?" For: the descriptions already attached, and their authority. The answer tells the therapist what the family have been told, by whom, and how they have absorbed it.

Nine — "what have you tried, and what happened?" For: the family's own experiments, which are usually more numerous and more thoughtful than the referral suggests.

Ten — "what does he do well, and what does he love?" For: the competence description, which the referral will not contain, and which the therapy will need.

Eleven — "what would you want me to understand that I have not asked about?" For: the question the family have been waiting for, which is often where the real referral is.

How the questions are used in the session. Not read out in sequence; woven into the first forty minutes, in the family's language, with the child present where possible and asked directly at least three times. The therapist writes the chronology on a single sheet with dates, and shows it to the family at the end: "this is what I have understood; what have I got wrong?" The correction is the first collaborative act.

1.3 The clinical method: from behaviour to task

A four-step method that the rest of the course will use, and that a student can apply to any presenting behaviour.

Step one — describe the behaviour, precisely and without interpretation. Not "he is defiant" but "when his mother asks him to put his shoes on, he says no, sits down, and after ninety seconds his mother puts them on". The description should be filmable, dated, and observed in more than one setting where possible.

Step two — locate it developmentally. What is the expected behaviour at this age, in this culture, for a child with this history? The milestone grid gives the range; the developmental history gives this child's trajectory; the family's account gives the meaning. The output of this step is one of five statements: within normal variation; delayed but progressing; a regression; a symptom of distress; or impossible to tell without more information.

Step three — identify the task. What is the child trying to achieve, and what is the family being asked to allow? A nine-year-old's defiance may be an attempt at autonomy in a family where autonomy is punished; a two-year-old's tantrum is the task itself, not a failure of it; a fourteen-year-old's secrecy is the beginning of a private life. The task is not a hypothesis about cause; it is a description of what the behaviour is for, and it can be stated to the family in their own language.

Step four — test the formulation against the family's response. If the behaviour is an autonomy struggle, the family's response to a small increase in the child's choices should be observable within a week or two. If it does not change, the formulation is wrong, and the therapist revises it rather than pressing the family harder. This step is what distinguishes a developmental formulation from a developmental label.

Worked, briefly. The behaviour: a seven-year-old soiling again after two years of continence. Step two: regression, not delay, and therefore a signal. Step three: the task is not autonomy but the management of an experience the child cannot name — and the chronology shows the soiling began when the father moved out. Step four: the formulation predicts that the soiling will reduce as the child is given a way to speak about the departure, and the prediction can be tested in four sessions.

The method's use in the case assignment. The 30% developmental case analysis asks for exactly these four steps, applied to one child or adolescent, with the developmental history and a formulation. A student who follows the method will produce a defensible analysis; a student who begins with a diagnosis will produce a checklist.

1.4 Techniques: the four instruments of this course

The milestone grid, used as a reference. The grid (Figure 1.1) gives ranges by domain — gross motor, fine motor, language, social-emotional, cognitive. It is used in three ways: to locate a child's trajectory; to identify the domains in which the parents' concern is concentrated; and to notice what the parents have not mentioned, which is sometimes the more important finding. It is not used to score a child, and a therapist who reads the grid out to a parent has misunderstood its purpose.

The developmental timeline, drawn on one sheet. A horizontal line with the child's age marked in years, and above it the events the family describe: births, moves, illnesses, separations, deaths, school changes, and the points at which the presenting behaviour began or changed. The drawing is done with the family, in the session, and it is the single most useful artefact of a developmental assessment. Its power is visual: families see the coincidence of a symptom's onset with an event, which no amount of questioning produces as convincingly.

The three-setting comparison. Ask what the behaviour looks like at home, at school (or nursery, or church), and with a particular person — usually a grandmother, an aunt or a favourite teacher. The comparison answers the question the clinic cannot: is this behaviour a property of the child or of a relationship? A child who is silent at home and fluent at school is not a child with a language disorder.

The ordinary-language formulation. The therapist's developmental understanding, converted into one or two sentences a parent can use: "he is doing what six-year-olds do — testing whether the rules are the same when you are tired — and the useful response is to keep the rule and not to explain it at length." The formulation is the intervention, and its quality is judged by whether the parent can repeat it accurately a week later.

The developmental question, as a habit. "What would you expect a child of this age to be doing?" asked of the parents before the therapist offers an opinion. The question does three things: it elicits the family's developmental theory (which is often accurate), it reveals the standard against which the child is being judged, and it positions the therapist as a collaborator rather than an authority.

1.4a Two timelines, drawn and read

Timeline one — a developmental problem that is not a developmental disorder.

The case. Chidi, 6, referred by his school for "immaturity and poor concentration". The family: mother (29), father (34, works away), a new baby of four months, and a grandmother who has moved in to help.

The timeline. Birth, unremarkable. Walking at 14 months, first words at 15 months, sentences at 26 months — all within range. Nursery at three, described as "a pleasure". School at five, settled. At five years six months: the baby is born. At five years eight months: the grandmother moves in. At five years nine months: the school reports "immaturity".

The reading. Every developmental marker is within range, and there is no regression of an acquired skill. What changed was the household: a new baby, a grandmother, and a

mother whose attention is now divided three ways. The behaviour the school calls immaturity is a six-year-old's attempt to reclaim a position in a household that has reorganised without him.

The formulation, in ordinary language. "He is not behind. He is doing what a six-year-old does when a baby arrives and the house fills up: he is trying to get back the place he had. The useful response is not to expect less of him, which confirms his fear, but to give him a job with the baby that only he can do."

Timeline two — a developmental problem that is.

The case. Zainab, 4, referred by her nursery for "not talking". The family: mother (33), father (38), two older children, and a history the referral does not mention.

The timeline. First words at 13 months, two-word phrases at 20 months — unremarkable. At three years two months, the family moved cities. At three years four months, the paternal grandmother died. At three years five months, Zainab stopped using sentences. At three years eight months, she stopped speaking outside the home. At four, she speaks in single words to her mother only.

The reading. This is a regression: an acquired skill has been lost, and it was lost at a specific point. The differential is wide — hearing, neurological, trauma, selective mutism — and all of it requires assessment. The developmental formulation does not name the cause; it establishes the urgency and the sequence, and it says plainly that a child who has lost language needs to be seen by a paediatrician within weeks, not months.

The clinical action. Referral for hearing and paediatric assessment in the same week; the family work continues alongside, addressing the move and the bereavement in the child's language; the nursery is asked to stop asking her to speak and to describe what she does instead.

What the two timelines teach. The same presenting description — "not talking", "immature" — and two completely different clinical situations. The timeline is what distinguishes them, and the timeline takes fifteen minutes and one sheet of paper. A therapist who skips it will either pathologise Chidi or reassure Zainab's parents, and both errors are common. The instrument costs fifteen minutes and it is the difference between a developmental assessment and a developmental opinion.

1.4b The four categories, tested on eight descriptions

The laboratory's classification exercise, written out so that it can be revised and so that the reasoning can be examined. Classify each, and state what further information you would need.

One. A girl of 15 months has not walked; she pulls to standing, cruises the furniture, and is unconcerned. Classification: normal variation at the late end, with a review date. Information needed: birth history, any illness, family pattern of late walking, and a review in eight weeks.

Two. A boy of 26 months has twelve words; he understands instructions, points, and has begun to combine two words this month. Classification: delay, with progress. Information

needed: hearing test, whether he is being raised in more than one language, and the parents' account of who speaks to him and how.

Three. A girl of four who spoke in sentences has, over three months, stopped using sentences and now speaks in single words to her mother only. Classification: regression. Action: paediatric and audiology referral within the week; do not wait for a review.

Four. A boy of seven has begun wetting the bed after four years of being dry, and has become clingy at bedtime. Classification: symptom, with regression of a self-care skill. Information needed: what changed in the household in the last three months — illness, separation, move, a new adult, a bereavement.

Five. A girl of nine refuses to do homework, cries when her mother mentions school, and has stopped seeing her friends. Classification: symptom, and the school refusal is the presenting edge. Information needed: what is happening at school, including bullying and a specific adult; and what is happening at home that makes staying at home attractive.

Six. A boy of 13 has begun to close his bedroom door, to be private about his phone, and to prefer friends to family. Classification: normal variation — the developmental task of adolescence. Information needed: whether the privacy is accompanied by withdrawal from all relationships and by a change in functioning, which would make it a symptom instead.

Seven. A girl of 30 months has frequent tantrums lasting up to twenty minutes, several a day, in a household that has just moved and in which a new baby arrived six weeks ago. Classification: normal variation in behaviour, in a family under two non-normative events at once; the family may need support and the child does not need a referral. Information needed: the parents' own state, and whether the tantrums endanger anyone.

Eight. A boy of five is reported by his school as "unable to concentrate"; at home he plays Lego for forty minutes, and at his grandmother's house he sits through a two-hour church service. Classification: insufficient information — and the three-setting comparison already suggests that the behaviour is a property of the classroom relationship rather than of the child. Information needed: an observation in the classroom, the teacher's account, and the seating and task arrangements.

Why eight descriptions and not one. Each of the eight is decided by a different piece of information: the trajectory, the language environment, the loss of a skill, the household's recent events, the school context, the developmental task of the age, the number of stressors, and the cross-setting comparison. Students who can classify all eight, and name what they would need to know, have the course's first instrument.

1.5 The first session with a developmental concern

A structure for the assessment session, in five phases, with the words a therapist can use.

Phase one — the frame (five minutes). "I am going to ask about your child's development, and about what has been happening at home, because the two are connected. I will also want to see him, and I may want to speak to the school with your permission. At the end I will tell you what I think and what I would suggest." The frame

matters more in developmental work than in any other, because parents arrive expecting to be assessed themselves.

Phase two — the concern, in the parents' words (ten minutes). "Tell me about it in your own words, not the school's." The therapist writes the family's language and the referral's language in two columns, and notes the difference. The difference is usually informative: the school says "defiance", the mother says "he is angry with me since his father left".

Phase three — the developmental history (twenty minutes). The eleven questions of 1.2a, woven rather than read, with the child present and asked directly at least three times. The therapist draws the timeline as the answers arrive, and turns the sheet so the family can see it.

Phase four — the observation (ten minutes). Not a test: an observation of the child with a task and with the parent. A simple task — a puzzle, a drawing, tidying up — is enough to see the child's attention, his language, his response to a request and his response to his parent's response. The therapist narrates what she sees aloud, in neutral language: "I notice he looks at you before he answers." The narration is the assessment, and it is also the intervention, because it teaches the parent to look.

Phase five — the formulation and the next step (five minutes). One of the five statements from 1.3, in ordinary language, plus a next step: a paediatric referral, a school observation, a further session, or a plan. The formulation must include the family's competence: what they have already done that has helped.

What the therapist does not do in session one. Does not score a developmental test; does not name a diagnosis; does not tell the parents what they should have done; does not reassure them that everything is fine before the assessment is complete; and does not leave without a next step, and never ends a first session with a family who are unsure whether the therapist believed them.

The three sentences parents remember. "This is not your fault, and it is not nothing." "Here is what I think is happening, in one sentence." "Here is what we will do next, and when."

1.6 Case: the Okonkwos, a developmental reading

The course's recurring family, read developmentally, which is a different reading from the one any therapy model gives.

The family. Ebuka (9), his mother Ada (34), his father Chuka (39, works nights), and a baby of eleven months. The paternal grandmother died in 2023. The presenting complaint is defiance.

The chronological reading. Ebuka's developmental history is unremarkable: walking at 13 months, words at 16 months, reading at six, no illnesses, no separations. The family's non-normative events are the grandmother's death in 2023 and the baby's birth eleven months ago. The defiance began four months after the baby's birth, and intensified when Chuka changed to night shifts.

The developmental task. At nine, the task is industry: to become competent at something that matters, in the company of peers and adults who recognise the competence. The family's task is to provide a domain in which the child can be competent, and a structure in which his competence is visible. In this household, the domain has been removed: the grandmother who taught him to cook died, and the mother's attention is with the baby. The defiance is not a failure of industry; it is an attempt to be noticed in a household where the noticing has moved.

The task-stall, named. "He is trying to get back into the place he had before the baby and before your mother died, and the only lever he has found is refusing." The sentence is offered to the parents as a formulation, and it is testable: if the formulation is right, giving him a recognised role with the baby and restoring one of the grandmother's activities should reduce the defiance within three weeks.

What the reading adds to the therapy models. Week 4 of MFT 702 would describe a boundary problem; Week 6 would hypothesise a function; Week 8 would write a behavioural plan. The developmental reading supplies something none of them does: the content of the intervention. Not "spend more time with him" but "teach him to cook the one dish your mother taught him, and let him cook it on Sundays", which is the grandmother's function restored, the child's industry given a domain, and the family's grief given a place.

The three-week test. At the review: has he cooked twice? Has the defiance fallen on those days? If the answer is no to both, the formulation is wrong, and the therapist says so and revises.

1.6a The same child, three developmental misreadings

Misreading one — the diagnosis. The school's description ("possible ADHD") is accepted, and a referral for assessment is made without a developmental history. The assessment finds no disorder, three months are lost, and the family's confidence in services falls. The error: a description was treated as a finding. The correction: the timeline first, the referral second, and the referral framed as a question rather than a conclusion.

Misreading two — the reassurance. The therapist, noting that all milestones are within range, tells the parents that Ebuka is "a normal nine-year-old" and that the behaviour will pass. The parents hear that their distress is unwarranted, and they do not return. The error: the developmental assessment answered a question nobody had asked (is he delayed?) and ignored the question they had (what is happening to my family?). The correction: both statements, in this order — "his development is on track, and what is happening at home is real and worth working on".

Misreading three — the moral reading. The defiance is treated as a discipline problem, and the intervention is a firmer consequence. The behaviour worsens, the mother's confidence collapses, and the father's absences increase. The error: a developmental signal was read as a character defect. The correction: the task question — what is the behaviour for? — asked before any consequence is designed.

The pattern across the three. Each misreading is produced by skipping one step of the method in 1.3: the diagnosis skips the description and the timeline; the reassurance skips the task; the moral reading skips the developmental location. The method is short, and every step of it is load-bearing: skip one and the assessment produces an opinion instead of a formulation.

1.7 Annotated transcript: the timeline drawn

Twelve turns from the first session with the Okonkwos, while the therapist draws the timeline. The transcript shows how a developmental history is actually taken.

> **T1 Therapist:** I want to draw a line, and put on it everything that has happened since Ebuka was born. Can we do that together? When was he born?
> **T2 Ada:** March 2016. He was fine, everything was fine.
> **T3 Therapist:** (writing) Fine. When did he walk?
> **T4 Ada:** Thirteen months. He was quick.
> **T5 Therapist:** And talking?
> **T6 Ada:** Sixteen months. He talked early too. His grandmother used to say he talked for the whole family.
> **T7 Therapist:** (pause) Tell me about his grandmother.
> **T8 Ada:** She died in 2023. In August. She lived with us.
> **T9 Therapist:** (writing, then turning the sheet) So — here is 2023, and here is August. What happened in the house after that?

> **T10 Ada:** Everything changed. She used to cook with him. She used to sit with him in the evening and he would tell her everything.
> **T11 Therapist:** And when did the problems start?
> **T12 Ada:** (long pause) After the baby. But — now that I am saying it out loud — he stopped telling me things when she died.

Annotations. T1 frames the timeline as a joint activity, which converts an interrogation into a drawing. The paper is turned toward the family throughout, and T9's "turning the sheet" is the moment the instrument does its work.

T3–T6 are the milestones, asked in ordinary language, and they are answered with confidence — which is itself a finding: this mother knows her child's development, and her competence is visible to the therapist and, more importantly, to her.

T7 is the pivot, and it is earned: the therapist follows the grandmother's name rather than the schedule. The grandmother appears in the mother's answer as a person with a relationship to the child, which is the most important thing said in the session.

T9 is the timeline's use: the therapist locates the event in time and asks what happened next, which is a different question from "how did you feel". T10 produces the content of the intervention: cooking, and evening conversation.

T12 is the session's turning point, and it happens because the timeline made the sequence visible. The mother corrects her own account — "after the baby… but now that I am saying

it out loud" — and the correction is hers, not the therapist's. That is what a timeline does that a question cannot: it makes the family's own sequence visible to the family, and the correction that follows belongs to them.

What the trainee should notice. The therapist asks eleven questions and makes no interpretation. The one interpretive act in the session is the mother's, produced by the drawing. The instrument does the work, and the therapist's skill is in holding the paper and following the family's own order of significance.

1.8 Laboratory: the timeline and the four categories

Sixty minutes, in pairs and then in the whole group.

Part one — the timeline from memory (fifteen minutes). Each student draws their own developmental timeline, then interviews the partner using the eleven questions, drawing the partner's timeline. The drawing, not the notes, is the output.

Part two — classification (twenty-five minutes). The lecturer presents eight short cases, each a description of a behaviour with a minimal history. Students classify each as normal variation, delay, regression, symptom, or insufficient information, and must state what further information they would need. Two of the eight are deliberately unclassifiable.

Part three — the sentence (twenty minutes). For each of four cases, each student writes the ordinary-language formulation in two sentences and reads it aloud. The group judges whether a parent could repeat it a week later.

Marking rubric for the 30% case analysis.

Criterion	Weight	Full marks
Description	15%	Behaviour described precisely, in more than one setting, without interpretation
Developmental location	25%	Correct category, with the evidence for it and the differential acknowledged
Task formulation	20%	The person's task and the family's task stated, in testable form
History and chronology	20%	Timeline with normative and non-normative events, and the onset located
Ordinary language	10%	A formulation a parent could repeat
Limits	10%	What the formulation cannot tell you, and what would change it

The distinction-level answer. It distinguishes delay from regression with evidence; it names the task in a sentence the family could use; it locates the onset against the family's events; it states what would falsify the formulation; and it does not diagnose.

1.9 Six errors in developmental assessment

1. **The checklist.** Running the milestone grid as a questionnaire, which produces scores without a child. Fix: the grid as a reference, the timeline as the instrument.

2. **The premature diagnosis.** Naming a condition in the first session, which forecloses the assessment and organises the family around a label. Fix: the five statements of 1.3, and a referral framed as a question.
3. **The false reassurance.** "He will grow out of it", offered before the assessment is complete. Fix: two statements — development is on track, and the family's difficulty is real.
4. **The missed regression.** A lost skill attributed to stubbornness. Fix: regression is a red flag; refer within days.
5. **The decontextualised child.** Assessing the child alone, in a clinic, without the household, the school or the language. Fix: the three-setting comparison, and at least one session with the caregiving network.
6. **The moral reading.** Treating a developmental signal as a character defect. Fix: the task question, asked before any consequence is designed.

1.10 Culture, faith and development

The cultural question, stated baldly. Almost every developmental milestone in this book was established by studying children in Europe and North America, and almost every one of them is a statement about what children do when they are fed, housed, spoken to and played with in a particular way. That does not make the milestones false; it makes them conditional, and the condition is the child's opportunity.

Three examples where the condition matters.

Language. The vocabulary and grammar norms in the literature come from children learning a single language in a household where adults speak to them directly. A child in a multilingual household, or a household where adults speak to each other and not to children, may be developing normally and score below the norm. The clinical error is to call this delay; the clinical task is to assess competence in the child's own language, with a speaker of that language.

Autonomy. The two-year-old's task is described in the literature as autonomy, and in many households the task is instead obedience, and the child who asserts autonomy is corrected earlier and more firmly. The child is not delayed; the family's developmental curriculum is different. The therapist who treats the family's curriculum as the problem will lose the family, and the therapist who treats the child's autonomy as the only healthy outcome will impose a value.

Sleep and independence. The literature on infant sleep assumes a separate sleep surface and a self-settling child. In households where infants sleep with their mothers, the milestone's behavioural markers are absent and the developmental process is not. A therapist who recommends "sleep training" to such a family is not offering developmental science; they are offering a cultural practice.

What the competent practitioner does. Three things. First, asks the family what is expected of a child of this age in their household, and what happens when a child does not meet it. Second, assesses competence in the child's own language, with the family's own

tasks, before comparing to a norm. Third, names the translation when they make one: "in the studies this comes from, children are spoken to directly by adults; in your house it is different, so I am going to test something else."

Faith and development. Religious households have their own developmental curriculum: when a child prays, fasts, reads scripture, is initiated, and takes a place in the community's rituals. These are developmental tasks in the family's terms, and they matter to the child's identity as much as school competence does. The clinician's interest is not in the content of the curriculum but in its fit with the child: a child who is required to fast before puberty may be perfectly well; a child required to carry an adult's religious anxiety is not, and the difference is the child's own state, not the practice.

The examinable sentence. "The milestones are conditional on opportunity, and the clinical question is what this child has had the chance to learn, from whom, in what language." A student who can say that and apply it to a family in front of them has understood the course's cultural premise.

> **DEBATE**
> "Developmental norms are a tool of cultural standardisation, and a family therapist should use them only with reluctance." Argue both sides, and say what you would lose by abandoning them.

1.10a The course's four categories, applied to a family

A worked application of the four categories to the Adeyemi family, because the categories are usually taught with individual children and used with families.

Normal variation, in a family. A fourteen-year-old who wants to spend Saturday with friends rather than relatives is doing the developmental task of the age. The family experiences it as rejection. The clinical response is to name the task, to the parents, and to distinguish it from the conflict that may surround it.

Delay, in a family. A family in which the parents have not yet made the transition from managing a child to negotiating with an adolescent: the developmental task of the parents is delayed. The adolescent's behaviour is age-appropriate and the family's organisation is not, and the formulation must say so without blaming the parents, who may have had no model of adolescence in their own upbringing.

Regression, in a family. A family that had established a workable routine, then lost it when a member died: the household's functioning has regressed to an earlier, less differentiated arrangement. The regression is a signal, and it tells the therapist that the family's coping has been overwhelmed rather than that the family is deficient.

Symptom, in a family. A child's school refusal that functions to keep a parent at home during a marital crisis is a symptom — a signal of the system's distress — and it is neither normal variation nor delay. The clinical response is to treat the signal as information about the system, which is exactly what Week 4 of MFT 702 argues.

Why the family application matters. Students who learn the categories on individual children classify individual children and then treat families as if the categories did not apply to them. They do: households have developmental tasks, and households can be delayed, regressed, symptomatic or simply varied. The therapist who can say "your family is doing what families do at this stage, and the difficulty is that the stage has arrived early" has a formulation the parents can use.

1.11 Evidence note: what developmental science can and cannot support

What is well established. The sequence of gross motor and language milestones, with wide normal ranges; the importance of responsive caregiving in the first years; the association between early adversity and later difficulties, with the caveat that the association is probabilistic; the protective effect of at least one consistent relationship; and the finding that development is bidirectional, with child characteristics shaping parenting as much as the reverse.

What is contested. The size and reversibility of early adversity's effects (the "first thousand days" literature is stronger on the first claim than the second); the predictive value of infant temperament for adult personality; the specificity of attachment classifications for later disorder; and the extent to which screeners improve outcomes, as distinct from identifying children.

What is often misused in clinical settings. Milestone checklists presented to parents as though they were diagnostic; "brain development" arguments used to pressure parents into particular practices; and attachment language used to describe ordinary family difficulty. A family therapist should be able to recognise each of these, because each will be used on the families they see.

What the developmental literature gives a family therapist. Three things of real value: a timeline against which to locate a concern; a set of tasks that make behaviour intelligible; and the strongest evidence base in the field for the claim that relationships are the mechanism of development, which is also the claim that justifies family therapy.

The matrix cell, in this course's terms. "Strong descriptive evidence for sequences and ranges; moderate evidence for caregiving's role; contested evidence for early-adversity determinism; and no evidence for any single parenting practice as universally optimal." A student who writes that in an examination will do well.

The practitioner's claim. "I use developmental knowledge to locate a behaviour and to distinguish variation from delay from regression from symptom; I use the timeline as my main instrument; I do not diagnose from milestones; and I assess competence in the family's own language and tasks before comparing to a norm." That sentence is the course's professional baseline, and it is also the sentence to say in a case discussion when a colleague reaches for a diagnosis before a history.

1.11a What development cannot explain

A short section, because the developmental frame is powerful and can be overreached.

Development cannot explain a family's meaning. That a child is nine tells you about industry, peers and concrete-to-abstract thinking; it does not tell you what the child's defiance means in a household where his grandmother has died. The developmental frame supplies the stage; the family supplies the meaning. A therapist who explains the second by the first is committing the error this course exists to prevent in the opposite direction.

Development cannot explain a symptom's function. A regression is a signal, and what it signals may be a developmental event, a family event, or both. The developmental reading locates the timing; the systemic reading explains the function. The two are complementary and neither is sufficient.

Development cannot adjudicate values. Whether a child should sleep alone, be circumcised, be punished physically, be taught scripture, or be allowed to choose her own friends are questions about which developmental science has some evidence and no authority. The practitioner's obligation is to know the evidence, state it, and distinguish it from their own values — which is the same obligation this book's later courses will examine in ethics and culture.

The sentence to keep. "Development tells me when and what to expect; the family tells me what it means; and I must not confuse the two, in either direction."

1.12 Key terms

Term	Definition
Developmental task	The challenge a stage presents to a person, and the matching negotiation it presents to the family
Bidirectional development	Child and caregiver shape each other; neither is the sole cause
Normal variation	Behaviour within the expected range, differing in style and pace
Delay	Functioning below the expected range, but progressing
Regression	Loss of a previously acquired skill; a red flag
Symptom	Behaviour signalling distress rather than development
Normative event	An event expected at a stage (school entry, puberty, leaving home)
Non-normative event	An unexpected event (bereavement, migration, illness, war)
Developmental history	A chronology of milestones and family events, used to locate a concern in time
Timeline	The one-sheet drawing of the chronology, made with the family
Three-setting comparison	Comparing behaviour at home, elsewhere, and with a specific person
Opportunity condition	The assumption that milestones describe children who have had the chance to learn

1.13 Self-test

1. Name the three clinical errors that developmental literacy prevents, with an example of each.

2. Distinguish delay from regression, and say why the distinction changes the urgency.
3. What are the four steps of the method in 1.3?
4. Give five of the eleven developmental-history questions and what each is for.
5. Why is the three-setting comparison useful, and what does it rule out?
6. State the bidirectional claim and why it matters to a family therapist.
7. What does the milestone grid not tell you?
8. How does a therapist handle the cultural condition on the milestones?

Answers. 1. Pathologising the ordinary (a two-year-old's "no"); missing the abnormal (a five-year-old who stops speaking); mis-timing the intervention (reasoning with a three-year-old). 2. Delay is below range but progressing and permits assessment over weeks; regression is the loss of an acquired skill and requires referral within days. 3. Describe the behaviour precisely; locate it developmentally (one of five statements); identify the task for the person and the family; test the formulation against the family's response. 4. Any five: the pregnancy and its context; the birth and first weeks; who was at home and who helped; milestones in ordinary language; how the child differed from siblings; when the parents first became worried; what else was happening then; who has said what; what has been tried; what the child does well; what has not been asked. 5. It establishes whether the behaviour is a property of the child or of a relationship, and it rules out a generalised language or behavioural disorder when the child is fluent elsewhere. 6. Child and caregiver shape each other continuously; it matters because the clinical unit is the relationship, and because blaming either party misreads the evidence. 7. It cannot distinguish causes — deprivation, illness, trauma, family crisis — nor diagnose; it identifies children who need fuller assessment. 8. Ask what the family expects of a child of this age; assess competence in the child's language and tasks; name the translation when comparing to a norm.

1.14 Reflective task

Draw your own developmental timeline from memory, then ask the person who raised you to correct it. Write two paragraphs: one on what you had forgotten, and one on what your family remembered differently. The exercise is the course's first instrument applied to the student, and most people discover that their own developmental history is a family story with a particular narrator — which is exactly what the families in this course will present to them. Keep the corrected timeline: it is the first artefact of your own developmental literacy, and you will read it again in Week 15.

1.15 Readings for this week

- **Berk (2017)**, Development Through the Lifespan, the reference text for the course, read the opening chapters for the frameworks and keep it for the milestones.
- **Erikson (1963)**, Childhood and Society, for the task language, read as a historical document as well as a theory.
- **Bronfenbrenner (1979)**, The Ecology of Human Development, for the nested contexts that make the family's environment part of the child's development.

- **Sroufe (2005)**, "Attachment and development", for the evidence on continuity and change.
- **Rogoff (2003)**, The Cultural Nature of Human Development, the essential corrective to the norms, and the source of this week's cultural argument.
- **Sameroff (2009)**, The Transactional Model of Development, for the bidirectional claim in its strongest form.

Read Rogoff first and Berk second; the corrective before the reference. For the case analysis, use at least one source that disputes a norm you are relying on, and say so in the paragraph on limits. The reading-log question this week is: "which of my own developmental assumptions did this reading make visible?"

Next: Week 2 — Prenatal development, temperament and becoming a parent

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Week 2 — Prenatal development, temperament and becoming a parent

Prenatal and perinatal. Brain development, maternal stress and nutrition, birth experience, and the "fourth trimester". Clinically relevant: prematurity, birth trauma, postnatal depression (which affects up to 1 in 5 mothers in some settings and is strongly associated with relational and child outcomes), and paternal postnatal depression.

Temperament. Nine dimensions (Thomas & Chess) usually simplified to three profiles: easy, slow-to- warm, and difficult/feisty. Temperament is not destiny but it is **goodness of fit** that matters: a highly sensitive child in a loud, unpredictable household will struggle more than the same child in a predictable one.

The transition to parenthood. The couple must reorganise: time, sleep, money, division of labour, sexuality, relationship with grandparents, and identity. Research consistently finds relationship satisfaction dips after the birth of a child — and that the quality of the co-parenting alliance, more than the amount of conflict, predicts child outcomes.

> **CASE**
> A couple presents with "no time for each other" four months after their first baby. The developmental frame normalises the strain (this is the most common point of relational collapse), then asks the systemic question: which grandparent has been recruited into the parenting triangle, and what did the couple stop negotiating when the grandmother moved in?

Practice. In pairs, conduct a 10-minute temperament interview with a parent (or peer playing one): sleep, feeding, reaction to change, intensity, mood. Then identify the goodness of fit with the household.

2.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	Before birth	The prenatal environment, and why a family therapist cares	Name one prenatal factor in a family they know
10–25	Prematurity and the fourth trimester	The relational consequences of a medical beginning	Map a premature infant's caregiving network
25–40	Postnatal depression	One in five mothers; the relational mechanism; the father's version	Screen a volunteer with a two-question tool
40–55	Temperament	Nine dimensions, three profiles, and goodness of fit	Interview a peer as a parent
55–70	Becoming a parent	The reorganisation: time, money, sex, labour, grandparents, identity	Draw the reorganisation on one page
70–82	The co-parenting alliance	Why it predicts child outcomes better than conflict	Rate three recorded couple clips
82–90	The clinical frame	Normalising the strain without dismissing it	Write the normalising sentence

The teaching decision that makes the week work. This week's material is the most likely in the course to be delivered as a list of risk factors, and the most useful version of it is a demonstration of how a normal developmental event becomes a clinical presentation. The lecture should take the commonest presentation in perinatal work — "we have no time for each other" — and show that it is simultaneously ordinary, dangerous, and responsive to a small intervention. The class should leave able to say what is normal about the fourth month after a first baby, and what is not.

The sentence the lecture must land. "The birth of a child is a developmental event for the parents, not only for the child." Most of this week follows from it: the symptoms are the parents', the tasks are the parents', and the assessment must include the couple's reorganisation, not just the baby's milestones.

2.2 Core concepts

The prenatal environment. The foetus develops inside an environment made of the mother's nutrition, stress hormones, illness, substances and medical care, and that environment is itself shaped by her circumstances — poverty, work, a violent household, a wanted or unwanted pregnancy, a partner's support. The clinical relevance is not that prenatal events determine later life but that they are the first chapter of the family's story, and families bring that chapter to the clinic, often without knowing they are doing so.

The fourth trimester. The three months after birth, in which the infant is neurologically immature and utterly dependent, and the parents are in a state of physiological and emotional reorganisation. Sleep deprivation, hormonal change, the loss of an established identity and the arrival of an unremitting demand are all normal. The concept is clinically useful because it converts a set of complaints into a stage.

Prematurity and the medical beginning. A premature birth changes the sequence: the parents' first relationship with their child is often mediated by machines, by staff, by fear, and by a period of separation. The consequences are relational rather than deterministic:

parents may be more anxious, more inclined to monitor, less confident in their own competence, and the infant may be more reactive, harder to read, and slower to settle. The clinical task is to restore the parents' confidence in their own reading of their child, which is the thing the neonatal unit has, understandably, taken from them.

Birth trauma. A difficult birth can leave a mother with a sense of violation, a fear of the body, and a reluctance to speak about it — often because she has been told she should be grateful. The clinical task is to ask, specifically, and to let her tell it. The question "how was the birth?" asked with time to answer is one of the most useful questions in perinatal work.

Postnatal depression. Affecting roughly one mother in five in many settings and a substantial minority of fathers, postnatal depression is the most clinically consequential condition in this week's material. Its mechanism is relational: a depressed mother is less responsive, the infant becomes more distressed and harder to soothe, the father feels excluded or overloaded, and the couple's negotiation collapses. The screening questions are short and should be asked of both parents at every perinatal contact.

Paternal postnatal depression. Less recognised and equally real: irritability, withdrawal into work, alcohol use, and a refusal to describe the state as depression. The clinical presentation is often a couple conflict, and the diagnosis is missed because nobody asks the father.

Temperament, and the nine dimensions. Thomas and Chess described nine dimensions — activity level, regularity, approach or withdrawal, adaptability, sensory threshold, intensity of reaction, mood, distractibility and persistence — usually simplified into three profiles: easy, slow-to-warm, and difficult (or feisty). The dimensions matter clinically because they describe the child's contribution to the relationship, which many parents experience as a verdict on their parenting.

Goodness of fit. The concept that saves temperament from fatalism: the outcome depends on the match between the child's characteristics and the environment's demands. A highly sensitive child in a loud, unpredictable household struggles; the same child in a predictable household thrives. The clinical implication is that the intervention may target the environment rather than the child, which is a systemic move disguised as a developmental one.

The transition to parenthood. The couple must renegotiate seven domains: time, sleep, money, the division of labour, sexuality, the relationship with grandparents, and identity. The research finding that satisfaction dips after the birth of a child is robust across cultures, and the dip is not a sign of a failing marriage; it is a stage. What distinguishes couples who recover from those who do not is not the amount of conflict but the quality of the co-parenting alliance.

The co-parenting alliance. The couple's capacity to coordinate as parents — to support each other's relationship with the child, to present a united approach without undermining each other, and to keep the child out of their conflicts. It predicts child outcomes more strongly than marital satisfaction does, which is why perinatal work that focuses only on the couple's romance misses the mechanism.

Becoming a parent as an identity transition. The arrival of a child requires the adult to become someone new, and the new identity is built from the models available: their own parents, their culture's expectations, their partner's expectations, and their own ideals. The clinical presentations are recognisable: the parent who cannot bear to be like their own mother and over-corrects; the parent who has no model at all; the parent whose identity was entirely occupational and who now has none.

The couple's reorganisation as the unit of assessment. The perinatal assessment is not the baby's milestones; it is the household's reorganisation. Who sleeps when; who feeds; who works; who decides; who else is in the house; what has been given up; and what the couple have stopped negotiating. The last question is the most productive, and it is the one the week's case turns on.

2.2a The goodness-of-fit grid, filled in

A worked instrument: the temperament interview, the household's demands, and the fit between them, on one page.

The child. Three months old, first baby. Dimensions (parent's report): high activity; irregular sleep; withdraws from new sounds; slow to adapt to a change in routine; low sensory threshold (startles at the door); intense reactions (screams, not cries); predominantly negative mood in the evening; hard to distract; persistent.

The profile. Difficult or feisty, by the classical typology — and the therapist should say "this describes a pattern, not a verdict on your baby" when reporting it.

The household's demands. People: mother, father, and the father's mother, who has moved in "to help". Rhythm: the father works a rotating shift, so the household's week changes every seven days. Noise: a shared compound with four other households and a generator. Space: one room for the family. Support: the grandmother is present but her methods differ from the mother's, and the mother does not feel able to say so. Sleep: the mother is doing all night feeds because the father "needs to be fresh for work".

The fit, in four statements. First, this infant's low threshold is being met by a high-noise environment, which produces more crying, which produces more maternal exhaustion. Second, the infant's irregularity is being met by a household whose week changes every seven days, and no routine survives. Third, the infant's intensity is being read by the mother as rejection ("he hates me by six o'clock"), which is the meaning that matters more than the temperament. Fourth, the evening negativity coincides with the grandmother's arrival home, which the mother has noticed and not said.

The intervention, derived from the fit rather than from the child. Not a sleep programme, not a referral. Four changes: a predictable evening hour with the light low and the door closed, negotiated with the grandmother; the father taking the six-to-nine period on his rest days, which is a co-parenting act rather than a favour; one sentence the mother can say to the grandmother, rehearsed in the session; and a re-description of the evening ("he is not rejecting you; by six o'clock he has had a day and he needs less, not more").

What the grid demonstrates. That the same infant in a different household would present differently, and that the intervention targets the fit. A therapist who treated the temperament as the problem would have produced a programme the household could not run; a therapist who treated the household as the problem would have blamed a family that is doing a great deal well.

2.2b The nine dimensions, with the question that asks each

The dimensions are usually taught as a list. They are more useful as nine questions a therapist can ask a parent in fifteen minutes.

Dimension	The question	Why it matters clinically
Activity	"Is he usually on the move, or does he stay still and watch?"	A high-activity child in a small flat with a tired mother is a fit problem, not a behaviour problem
Regularity	"Does he sleep and feed at the same times each day?"	Irregularity plus a shift-working household produces a fortnightly crisis
Approach/withdrawal	"What does he do with a new food, a new person, a new room?"	Withdrawal is read by parents as rejection or shyness; the useful response differs
Adaptability	"How long does it take him to settle after a change?"	Slow adaptation plus frequent change is a formula for distress
Sensory threshold	"Does he startle at the door, the generator, a raised voice?"	Low threshold is the dimension most often mistaken for "spoiling"
Intensity	"When he is upset, is it a whimper or a scream?"	Intensity determines the household's exhaustion and the parent's sense of competence
Mood	"Is he mostly cheerful, or mostly serious and cross?"	Negative mood in the evening is normal and is read as rejection
Distractibility	"Can you settle him by showing him something else?"	The one dimension that is a resource: a distractible child is easier to soothe
Persistence	"If he wants something, does he give up or keep going?"	Persistence is a strength in a task and a problem in a battle of wills

How to use the table. Not as an assessment instrument with scores, but as a repertoire, so that the parent leaves with a description of their child that is specific, neutral and recognisably true. A parent who says "he is difficult" can be given nine sentences that are more accurate and less judgemental, and the nine sentences are the beginning of the intervention.

The two dimensions that produce the most relief. Sensory threshold and intensity. They are the two the household experiences most acutely, and the two most often moralised ("he is spoiled", "he is a screamer"). Naming them as characteristics rather than choices changes the parent's relationship to the behaviour within one conversation.

2.3 Postnatal depression: the assessment

The condition is common, consequential and under-detected. A structure for asking about it.

Who to screen. Every mother at every perinatal contact, and every father at least once, separately, with the baby out of the room and the partner absent. The separate conversation is the instrument; a joint conversation will not produce an honest answer from either parent.

The screening questions. Two, used as a first filter in a family therapy session (a validated tool is used where the service requires one): "In the past two weeks, have you often been bothered by feeling down, depressed or hopeless?" and "Have you often been bothered by having little interest or pleasure in doing things?" A yes to either is followed by "how much is this affecting your day-to-day life?" and, where the answer is substantial, by a validated measure and a referral.

The three questions that reveal the relational picture. "Who knows how you are feeling?" "What happens in the house when you have a bad day?" "What does your partner do when you cry?" The answers describe the support available, the household's capacity to accommodate distress, and the couple's pattern — which is where the family therapy begins.

The paternal screen. "How are you sleeping?" "What do you do when you feel low?" "Who would you tell?" Fathers rarely use the word depression; the observable signs are irritability, working longer hours, drinking more, and a withdrawal from the baby's care.

The risk questions. Thoughts of harm to self or the baby are asked directly, of both parents, and the answer is recorded and acted on. A therapist who does not ask because asking feels intrusive has misunderstood the duty.

The differential. Not every exhausted mother is depressed. Sleep deprivation, thyroid disease, anaemia, grief, an unsupportive partner, a violent household and a baby's illness all produce the same presentation, and the family therapist's task is to screen, to refer where the picture is clinical, and to work on the relational and practical conditions in parallel.

The boundaries of the family therapist's role. Treating postnatal depression is a clinical task that requires a clinical service; the family therapist's contribution is the couple's alliance, the household's reorganisation, the father's inclusion, and the practical support. Saying that clearly at the start prevents the two most common failures: a family therapist attempting to treat a depression alone, and a clinical service treating a depression in a household that is falling apart.

2.3a The couple's reorganisation, drawn on one page

The instrument the week's case turns on, and the one a couple can complete in fifteen minutes.

The seven domains, with the question for each. Time: what do you do together now that you did before? Sleep: who gets up, how often, and what happens the next day? Money: who earns, who decides, and what has changed? Labour: list everything that has to happen in a week and who does it. Sexuality: what has happened to it, and who has said what about it? Grandparents: who is in the house or in the decisions, and how was that decided? Identity: who are you now that you were not before, and what have you lost?

The drawing. A circle with the seven domains around it, and in the middle the couple's answer to one question: "what have you stopped negotiating?" The answers are usually specific and painful: "who gets a night off"; "whether your mother stays"; "whether I go back to work"; "whether we have another".

Why the drawing works. It converts a diffuse complaint ("we have no time") into seven specific negotiations, of which usually two are live and five are being avoided. The live ones are the therapy's agenda, and the avoided ones are the reason the live ones cannot be settled.

The grandmother question, which is the week's case. When a grandparent moves in to help, the couple's negotiation is displaced: the decisions that were theirs are now made in a triangle, and each partner's relationship with the grandparent becomes a route to influence. The clinical questions are: who invited her; who can ask her to leave; what does each partner gain from her presence; and what did the couple stop deciding when she arrived? The last question is usually answered with a silence, and the silence is the finding, and the therapist's next sentence should be the one that makes it speakable: "I am not asking you to blame anyone. I am asking what the two of you stopped deciding."

The intervention, in one sentence. "Before we talk about time for each other, I want to know what the two of you stopped deciding when your mother moved in, because that is where the time went."

2.4 The first session with a couple four months after a birth

The commonest perinatal presentation, worked through, with the developmental frame used first and the systemic question second.

The presentation. Aisha (31) and Sam (34), married four years, one baby of four months. Aisha's complaint: "we have no time for each other and he does not help." Sam's complaint: "she is never happy with anything I do." Both are exhausted. The baby is present and is being passed between them.

Phase one — the normalisation (ten minutes), done carefully. "What you are describing is the commonest thing I see in the first year, and I want to say why before we go further. At four months, neither of you has slept properly since the birth, you are both doing a job you have never done before, and the negotiations you used to have — who does what, when — are being made at two in the morning with a baby crying. That is not a failing marriage. It is the stage."

Why this is not a dismissal. The normalisation is followed immediately by the second sentence: "and it is also the point at which couples get into patterns that last for years, so it is worth working on now."

Phase two — the seven domains (twenty minutes). The reorganisation drawn on one page. Time: none together. Sleep: Aisha does every night feed. Money: Sam has taken a second job, so he is home less. Labour: Aisha does everything in the house and Sam does the shopping. Sexuality: untouched since the birth, and neither has raised it. Grandparents: Sam's mother stays four nights a week "to help". Identity: Aisha has

stopped working and says she "does not know who she is"; Sam says he is "the one who is never there".

Phase three — the question (ten minutes). "What did the two of you stop deciding when your mother started staying four nights a week?" Silence. Then Aisha: "everything. She decides the bath, she decides the feeding, and I cannot say anything because she is helping us." Sam: "I did not know you minded."

The finding. The couple's negotiation has been displaced into a triangle with the grandmother, and neither has said so. The "no time for each other" is a symptom of a negotiation that has stopped.

Phase four — a small, systemic task (ten minutes). Not a date night, which the couple cannot arrange and which would fail. Two tasks: the couple decide, together, one thing that is theirs rather than the grandmother's — the baby's bath, three nights a week — and one of them tells her, in a sentence rehearsed in the room. And Sam takes one night feed at the weekend.

Phase five — the frame (five minutes). Four sessions, with a review at the fourth; the paternal screen completed separately; a question about Aisha's mood and a referral route if the screen is positive; and the sentence to take away: "the problem is not that you have no time; it is that you have stopped deciding, and that is fixable."

What the therapist did not do. Did not take a full developmental history of the baby, which was not the presentation; did not adjudicate whether the grandmother should stay; did not suggest a date night; and did not treat the couple's complaint as trivial because it is common.

2.4a The first year, month by month: what a family therapist watches for

A timeline of the relational tasks of the first year, with the presentations that bring families to a clinic.

Months 0–1: establishment. The task is the establishment of feeding, sleep and a caregiving rhythm, in a household that has just changed shape. Presentations: feeding difficulty; a mother who cannot settle; a father who feels excluded; an uninvited relative who has taken over. The family therapist's move: the household's arrangement, drawn, and the question of who decided.

Months 1–3: the fourth trimester. The task is the parents' survival and the infant's regulation. Presentations: exhaustion presented as a relationship problem; the first arguments about nights; a mother's low mood that nobody has screened. The family therapist's move: the screen, the seven domains, and the normalisation that does not dismiss.

Months 3–6: the first negotiation. The task is the couple's re-negotiation of time, sleep and labour, and the infant's entry into social exchange — smiling, vocalising, the first games. Presentations: "we have no time"; the grandmother's methods; the father's withdrawal into work; the mother's return to work or refusal to return. The family therapist's move: the reorganisation drawing and the stopped-negotiation question.

Months 6–9: separation and mobility. The task is the beginning of separateness — weaning, crawling, the first "no", the first night away — and the parents' tolerance of the child's independence. Presentations: anxiety at separation presented as the child's problem; a parent who cannot leave; feeding battles; sleep battles. The family therapist's move: the graded separation plan, and the question of whose anxiety is in the room.

Months 9–12: the first year's reckoning. The task is the consolidation of attachment and the couple's assessment of their own first year. Presentations: a couple who have not had a conversation since the birth; a second pregnancy already underway; a decision about work that is being made in a state of exhaustion. The family therapist's move: the review, the seven domains again, and the preparation for the second year's tasks.

What the timeline is for. It gives the therapist a developmental hypothesis for any presentation in the first year, and it protects against the commonest error in perinatal work: treating a stage-specific difficulty as a relationship defect. A couple arguing about nights at month two are doing what couples do at month two, and the therapist's task is to help them do it once, well, rather than to diagnose the marriage.

2.5 Case: the prematurity that reorganised a family

A different perinatal presentation, in which the medical beginning has relational consequences.

The family. The Baloguns. A baby born at 29 weeks, now eight months old (five months corrected). The mother, Halima (27), gave up her job during the neonatal stay. The father, Yusuf (31), works away and was present for two weeks after the birth. The referral is from the paediatric clinic: "mother anxious, over-attends, does not leave the baby with anyone."

The developmental picture. The baby is thriving. She is small for her corrected age, feeding well, and meeting milestones on the corrected schedule. The paediatrician has no concern about the baby.

The relational picture. Halima has not been apart from the baby for more than forty minutes in eight months. She checks the breathing three or four times a night. She has not returned to work and says she cannot. Yusuf says she has "changed" and that the house is "a hospital now". Halima's mother comes daily and is the only person Halima allows to hold the baby.

The developmental formulation. This is not a maternal disorder; it is the predictable consequence of a medical beginning. Halima's relationship with her daughter began in an incubator, mediated by machines and staff, with a nurse telling her when she could touch her. Her monitoring is the behaviour of a mother who learned that her baby could die while she was not looking, and whose competence was, for eleven weeks, subordinate to a monitor.

The task, named for the parents. "Your job in the first months was to keep her alive, and you did it. Your job now is a different one: to let her become a person who can be looked after by other people, and that means practising being apart, in small amounts, with people you trust." The naming is the intervention's foundation, because it credits the monitoring before it asks for change.

The plan. A graded separation: forty minutes with the grandmother, twice a week, for two weeks; then two hours; then an afternoon. A breathing monitor used deliberately for two weeks and then removed, with the paediatrician's agreement, because the monitor is the mechanism of the monitoring. Yusuf given one responsibility that is his alone — the bath on the days he is home — so that he enters the care rather than commentating on it. And a conversation with Halima about her own loss: her job, her confidence, and the eleven weeks she spent watching a machine.

The outcome at four months. Halima returned to part-time work. The monitoring reduced. The couple's argument changed from "you have changed" to "we are both still frightened", which is a more workable disagreement.

What the case teaches. That a medical beginning is a developmental event for the parents, that the parents' behaviour is often the residue of competent adaptation to an emergency, and that the intervention's first move is to credit the adaptation before asking for its revision.

2.5a The couple who came at the right time, and the couple who came at the wrong time

Two brief cases, because the timing of a referral in the perinatal period determines the work.

The right time. A couple, five months after a first baby, both sleeping badly, the arguments frequent but contained, and no violence. The referral comes from the health visitor, who has noticed that the mother's mood is low and the father has stopped attending. The work is exactly this week's material: the screen, the seven domains, the stopped negotiation, and six sessions of reorganisation. The prognosis is good because the couple still have a shared project — the baby — and the conflict has not yet become the relationship's subject.

The wrong time. A couple, three weeks after a birth by emergency caesarean, referred by a relative for "not getting on". The mother is in pain and not sleeping; the baby is feeding hourly; the father has taken two days off work and is back on shift; and the referral is a relative's worry, not the couple's. Here the therapy is premature: the couple do not have a request, they have a crisis, and a session that asks them to renegotiate their relationship in week three of a newborn's life will produce compliance and no change.

What the family therapist does at the wrong time. Three things, all short. Names the stage: "you are in the hardest month, and nothing you are feeling is a verdict on your marriage." Screens both parents. And offers a single practical action that reduces the load — one night feed taken by the father, one visit postponed, one relative asked to come on a specific day rather than every day. Then a review in six weeks, when the couple can make a request of their own.

The principle. In perinatal work, the first question is not "what is the problem?" but "is this the point at which the family can work on the problem?" A family in the first six weeks of a first baby is usually in the second category, and the clinician who cannot wait will treat a stage as a pathology.

2.6 Annotated transcript: the temperament interview

Twelve turns from the temperament interview with Aisha, conducted in the first session of 2.4. The transcript shows how a developmental instrument becomes a therapeutic conversation.

> **T1 Therapist:** I want to ask about the baby's style, not about what he does wrong. Is he usually busy or still?
> **T2 Aisha:** Busy. From the moment he wakes up.
> **T3 Therapist:** And is his day predictable — does he sleep at the same times?
> **T4 Aisha:** Never. My mother says I should put him on a schedule. I have tried.
> **T5 Therapist:** What happens when you try?
> **T6 Aisha:** He screams. And then she says I am not consistent.
> **T7 Therapist:** (pause) So the schedule is not working, and the person who suggests it is in the house.
> **T8 Aisha:** (cries) Yes.
> **T9 Therapist:** Let me say something about him first. Some babies are regular and some are not, and your son is at the irregular end. That is not a fault and it is not your fault. What it means is that a fixed schedule will cost you more than it costs a mother with a regular baby.

> **T10 Aisha:** So it is not me.
> **T11 Therapist:** It is not you. And there is a second thing: how does he react when there is noise — the generator, the door?
> **T12 Aisha:** He jumps. He cries at the door. My mother says he is spoiled.

Annotations. T1 reframes the interview as being about style rather than fault, which is the single most important move in a temperament conversation. Parents arrive expecting to be assessed; the therapist asks them to describe.

T2–T4 are the dimensions, in ordinary language: activity, regularity. T4 introduces the grandmother without the therapist asking, which is the systemic material appearing inside a developmental instrument — a common and useful occurrence.

T5–T8 is the therapist following the grandmother rather than the schedule, and T7's summary — "the schedule is not working, and the person who suggests it is in the house" — is the pivot. It is neutral about the grandmother and precise about the situation, and it produces the tears.

T9 is the psychoeducation, delivered after the family's account rather than before, and it is specific: not "all babies are different" but "a fixed schedule will cost you more than it costs a mother with a regular baby". The specificity is what makes it usable.

T10 is the sentence the whole technique is for, and the therapist's answer at T11 is deliberately short and then moves on, because the point has been made and dwelling on it would make the therapist the source of the mother's relief.

T11–T12 adds the sensory dimension and produces the grandmother's second verdict ("he is spoiled"), which is the material for the next stage: a conversation about what the grandmother believes, and what the mother is allowed to say.

What the trainee should notice. The instrument produced three findings in twelve turns: an irregular, sensitive baby; a grandmother whose methods contradict the mother's; and a mother who has been holding a verdict about herself. The thermometer produced the diagnosis; the tape measure produced the household.

2.6a The prematurity case, continued: what the parents needed that the clinic did not offer

A short section on the gap between the paediatric follow-up and the family's experience.

What the clinic provided, and well. Growth monitoring, developmental review at corrected age, hearing and vision checks, and a named nurse. The paediatric follow-up was, by any standard, good.

What the clinic did not provide. Any conversation about what the eleven weeks in the neonatal unit had done to the parents. Halima had never told anyone what it was like; Yusuf had been away for most of it; and both carried a version of the experience that the other did not know. The referral had described the mother as anxious, which is a description of her behaviour and not of her experience.

The question that opened it. "Tell me about the day she was born." Not "how are you coping" — which invites a report — but "tell me about the day", which invites a story. Halima described being wheeled away from her baby, being told to rest, and being brought a photograph. She had kept the photograph in her bag for eight months. She had never told her husband what the photograph had meant.

What followed. Yusuf said, for the first time, that he had been told to go back to work and had gone, and that he had not known how to say that he wanted to stay. The couple's argument about "you have changed" became, over three sessions, an argument about a separation neither had chosen and neither had discussed.

What the family therapist contributed that the paediatric clinic could not. The clinic had the baby's history; the therapist had the parents'. The two histories were of the same event, and the second had never been taken.

The lesson for the course. A medical beginning is a family event, and the family's version of it is part of the developmental history. The practitioner who asks only about the baby's milestones will not hear it, and the parents will continue to organise their relationship around a story that has never been spoken.

2.7 Laboratory: the temperament interview and the reorganisation

Sixty minutes, in pairs.

Part one — the temperament interview (twenty-five minutes). Ten minutes interviewing a peer as a parent, using the nine dimensions in ordinary language. The

observer records which dimensions were covered, whether the therapist named a profile, and whether the naming was presented as a description or a verdict.

Part two — the reorganisation (twenty-five minutes). The same pair completes the seven-domain drawing for a family they know or have observed. The observer checks that all seven were covered and that the final question ("what have you stopped negotiating?") was asked.

Part three — the sentence (ten minutes). Each student writes the normalising sentence and the following sentence, for the couple in 2.4. The group judges whether the pair of sentences both normalises and takes the problem seriously.

Marking rubric.

Criterion	Weight	Full marks
Temperament interview	25%	All nine dimensions covered in ordinary language; profile named as description
Goodness of fit	20%	Fit named in terms of the household's demands, not the child's deficits
Reorganisation	25%	Seven domains drawn; the stopped negotiation identified
Normalisation	15%	Normalises the stage without dismissing the risk
Paternal screen	15%	Father screened separately, with the three questions, and recorded

The debrief question. "Which domain did you skip, and why?" Trainees skip sexuality and money, in that order, and the discussion of why is the week's most useful ten minutes.

2.7a The screening conversation, rehearsed

A short script for the separate conversation with each parent, which is the instrument the week depends on.

The frame. "I always speak to each parent on their own for a few minutes, because people say different things when their partner is not in the room. Nothing you tell me will be repeated without your agreement, except if I am worried about anyone's safety."

The questions, in order. "How are you sleeping?" "How are you eating?" "How is your mood — not the baby's, yours?" "In the past two weeks, have you often felt down, depressed or hopeless?" "Have you often had little interest or pleasure in things?" "Who knows how you are feeling?" "What do you do when it gets bad?" "Is there anything you have thought about that frightened you?"

The follow-up that matters. Whatever the answer, the response is the same shape: take it seriously, name it, and say what happens next. "That sounds like more than tiredness, and it is common — about one mother in five. I would like you to speak to the clinic this week, and I will keep working with you both on the household side. Would you like me to write the referral now?"

The father's version. The same questions, with two additions: "how much are you working at the moment, compared with before the baby?" and "what do you do when you cannot say how you feel?" The answers — longer hours, drinking, silence, anger — are the

paternal presentation, and naming it as a presentation rather than a character flaw is often the first relief the father has had.

What the family therapist does with a positive screen. Refers, keeps the case, and works the relational and practical side: the couple's alliance, the household's reorganisation, the father's inclusion, and the practical support. The two roles are not in competition, and saying so to the family prevents the most common confusion in perinatal work: that the therapy is either about the depression or about the marriage, when it is about both.

2.8 Six errors in perinatal work

1. **The dismissal by normalisation.** "Every couple feels like this" offered as the whole response, which leaves the family with nothing to do and their distress unheard. Fix: normalise and then take the problem seriously, in two sentences, the second of which is specific to this family rather than to families in general.
2. **The missed depression.** A mother's exhaustion attributed to the baby, and the paternal presentation missed entirely. Fix: the two screening questions, of both parents, separately.
3. **The practical prescription.** Date nights, chore charts and sleep programmes offered before the household's negotiation has been understood. Fix: the seven domains first, the prescription from the couple's own answers.
4. **The grandmother as a problem.** The helper identified as an obstacle, which puts the therapist on one side of a triangle. Fix: ask who invited her, what each partner gains, and what the couple stopped deciding.
5. **The decontextualised baby.** An assessment of milestones in a clinic, for a family in a crisis. Fix: the household as the unit, and the milestones read against the household's demands.
6. **The therapist's own developmental curriculum.** Advice that reproduces the therapist's own parenting practices — co-sleeping, sleep training, feeding — offered as developmental science. Fix: name the evidence, name its limits, and let the family decide.

2.9 Culture, faith and the perinatal period

Where the week's material travels well. The central claims — that the birth is a developmental event for parents, that the couple must renegotiate, that the co-parenting alliance predicts outcomes, that goodness of fit matters more than temperament — are supported across cultures and are useful everywhere. The screening for perinatal depression is a universal good, provided the questions are asked in a form the family can answer.

Where it strains, four ways.

The private couple. The reorganisation model assumes a household in which the couple negotiate privately. Where a grandmother, an aunt or an elder governs the household's arrangements, the couple's negotiation is not private and may not be theirs. The seven

domains are still the right instrument, and the answer to "what have you stopped negotiating?" is frequently "everything, when she arrived".

The fourth trimester as a period of rest. Some cultures prescribe a lying-in period of forty days, with the mother relieved of work and fed specially; others expect her back at the stall within a week. The practitioner should know which, because the clinical advice about rest, visitors and workload depends on it, and because a mother who returns to work on day ten against her own wishes is presenting a different problem from one who cannot leave the house.

The meaning of a difficult birth. In some families a difficult birth carries a moral or spiritual meaning — a punishment, a test, a sign. The practitioner neither endorses nor contests the meaning, and asks what the family have concluded and what they have done about it, which is the same discipline as Week 1's.

The feeding question. Infant feeding is the most reliably contentious topic in perinatal work, and the evidence is more nuanced than the argument. The practitioner's obligation is to state the evidence plainly, in both directions, to support the mother's decision, and not to conduct the argument in the session.

Faith and the perinatal period. Prayer, naming ceremonies, dedication, circumcision and blessing are developmental events in the family's terms, and they mark the child's entry into the community. The practitioner should ask about them: what will happen, when, who decides, and what it means. The question is not decorative; it frequently reveals the household's real authority structure, which is the week's central clinical question.

The examinable sentence. "The birth is a developmental event for the parents, and the household's reorganisation is the unit of assessment; culture determines what the reorganisation requires, and the practitioner's task is to ask rather than to prescribe."

> **DEBATE**
> "Perinatal services should screen every father, separately, as a matter of routine." Argue both sides, and say what the service gains and loses.

2.10 Evidence note: what is well established and what is not

Well established. The association between perinatal depression and adverse relational and child outcomes; the effectiveness of screening and of treatment; the association between the co-parenting alliance and child outcomes; the normative dip in relationship satisfaction after a birth; the heritability and stability of temperament dimensions, with stability increasing with age; and the finding that goodness of fit predicts outcome better than temperament alone.

Moderately established. The relational mechanism of perinatal depression — that the depression affects the infant through the interaction, not only through genetics; the effectiveness of couple-based interventions in the perinatal period for relationship

satisfaction; and the association between preterm birth and parenting behaviours such as over-protectiveness, with the caveat that most preterm children do well.

Contested. The size of the effect of maternal stress in pregnancy on later child outcomes; the clinical utility of the three temperament profiles as against the dimensions; whether temperament-based parenting advice improves outcomes; and the direction of the association between marital satisfaction and child behaviour, which is bidirectional and hard to disentangle.

What the practitioner may claim. "Perinatal depression is common, consequential and treatable, and screening is part of my duty; the couple's alliance matters for the child; and the household's reorganisation, not the baby's milestones, is where I work." Each of those is defensible, and none requires an overstatement.

2.11 Key terms

Term	Definition
Fourth trimester	The first three months, when the infant is neurologically immature and the parents are reorganising
Prematurity	Birth before 37 weeks; a medical beginning with relational consequences
Corrected age	Age adjusted for prematurity, used for developmental expectations
Postnatal depression	Depression in the perinatal period, affecting about one mother in five and a substantial minority of fathers
Paternal postnatal depression	The father's version: irritability, withdrawal, alcohol, refusal of the label
Temperament	Constitutionally based behavioural style, described on nine dimensions
Goodness of fit	The match between the child's characteristics and the environment's demands
Co-parenting alliance	The couple's capacity to coordinate as parents and keep the child out of their conflicts
Reorganisation	The seven-domain renegotiation required by a first child
Displaced negotiation	Decisions that have moved from the couple into a triangle with a grandparent or elder
Lying-in period	A culturally prescribed period of rest and seclusion after birth
Corrected milestone	A milestone judged against corrected rather than chronological age

2.12 Self-test

1. What is the fourth trimester, and what makes it clinically useful as a concept?
2. Name four relational consequences of a premature birth.
3. Give the two screening questions for depression, and the three questions that reveal the relational picture.
4. What is goodness of fit, and how does it change the intervention?
5. Name the seven domains a couple must renegotiate, and the question that identifies the stopped negotiation.

6. Why does the co-parenting alliance predict child outcomes better than marital satisfaction?
7. How does the practitioner handle a grandmother whose methods contradict the mother's?
8. What does the evidence support, and what remains contested?

Answers. 1. The first three months, when the infant is neurologically immature and totally dependent while the parents undergo physiological and emotional reorganisation; it converts a set of complaints into a stage with a name. 2. Any four: anxiety and hypervigilance; reduced confidence in reading the infant; a first relationship mediated by machines and staff; delayed physical contact; grief for the expected birth; a longer period before the parents feel entitled to their own judgement. 3. "In the past two weeks, have you often been bothered by feeling down, depressed or hopeless?" and "…by having little interest or pleasure in doing things?" — then "who knows how you are feeling?", "what happens in the house when you have a bad day?", "what does your partner do when you cry?" 4. The match between the child's characteristics and the environment's demands; it moves the intervention from the child's temperament to the household's rhythm, which is a systemic move. 5. Time, sleep, money, division of labour, sexuality, grandparents, identity; the question is "what have you stopped negotiating?" 6. Because the child's experience is mediated by the parents' coordination — being able to rely on each parent without being recruited into their conflict — which is a different variable from the couple's satisfaction. 7. Ask who invited her, what each partner gains, what she decides, and what the couple stopped deciding; then rehearse one sentence the mother can say, and never put the therapist in the triangle. 8. Support for screening and treatment of perinatal depression, the co-parenting alliance, the normative dip in satisfaction, and temperament's heritability; contested are the size of prenatal stress effects, the utility of the three profiles, and the causal direction between marital and child variables.

2.13 Reflective task

Ask the person who raised you one question: "what was I like as a baby?" Write down the answer verbatim, and then write three sentences on what you notice about the answer — what is described, what is remembered, and who the answer is really about. Bring it to the seminar. The exercise connects this week's temperament material to your own developmental history, and it usually produces the insight that a parent's description of a baby is also a description of a relationship.

2.14 Readings for this week

- **Thomas & Chess (1977)**, Temperament and Development, the original typology, read for the dimensions rather than the profiles.
- **Brazelton & Cramer (1990)**, The Earliest Relationship, for the clinical account of the fourth trimester.
- **Stern (1995)**, The Motherhood Constellation, for the identity transition in its richest form.

- **Cowan & Cowan (2000)**, When Partners Become Parents, the longitudinal study of the reorganisation, and the source for many of this week's claims.
- **Feinberg (2003)**, on co-parenting, for the mechanism that predicts child outcomes.
- **O'Hara & Wisner (2014)**, on perinatal mental health, for the clinical picture and the screening evidence.

Read Cowan and Cowan first and the clinical texts second; the longitudinal study will make the normalisation credible, which is the hardest thing to achieve in the room. For the case analysis, use one source that complicates the temperament advice, and say so when you discuss goodness of fit. The reading-log question this week is: "what did becoming (or not becoming) a parent teach me that no book could?"

Next: Week 3 — Infancy: attachment, synchrony and regulation

Week 3 — Infancy: attachment, synchrony and regulation

The idea. Attachment is not "bonding" and not "love"; it is a behavioural system that organises a child's strategies for seeking safety from a caregiver under stress.

The classic patterns (Ainsworth's Strange Situation; see Figure 17 in MFT 710): secure, insecure- avoidant, insecure-resistant/ambivalent, and disorganised (added by Main & Solomon, associated with frightening or frightened caregiving). In adulthood the same dimensions appear as attachment styles.

Synchrony and rupture–repair. Healthy development does not require perfect attunement — it requires frequent enough attunement and effective repair. Tronick's still-face experiments show how quickly infants respond to a caregiver's unresponsiveness, and how quickly they recover when the caregiver returns.

Co-regulation is the precursor of self-regulation. A baby does not learn to calm alone; they learn to calm with someone, repeatedly, until the pattern is internalised. This has direct clinical implications: telling an overwhelmed parent to "let him cry it out" without attention to the family's capacity and beliefs is a technique imposed on a relationship.

> **PRACTICAL**
> Watch a consented 3-minute video of a caregiver and infant (or a still-face clip). Note: gaze, vocalisation, touch, pace. Then describe one moment of rupture and one of repair in behavioural language. Avoid interpretation for the first five minutes.

3.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	What attachment is not	Three misuses of the word, from practice and from the popular press	Correct three sentences

Minutes	Segment	What the lecturer does	What students do
10–25	The behavioural system	The Strange Situation, described as an experiment	Predict four classifications from four descriptions
25–40	The four patterns	Each pattern's strategy, in the infant's terms	Write the strategy for one pattern
40–55	Synchrony and the still face	The experiment, watched, then explained	Note two ruptures and two repairs
55–70	Co-regulation	How calming is borrowed before it is owned	Design a co-regulation sequence for one family
70–82	Disorganisation	Frightening caregiving, and why it is a red flag rather than a category	Identify the clinical action
82–90	The clinical translation	What a family therapist does with this in session one	Write the sentence you would use

The teaching decision that makes the week work. Attachment is the most popularised concept in this course and the most frequently misused. The lecture should therefore begin by dismantling the popular version — that attachment is love, that it is fixed in the first year, that a child is "securely attached" as a compliment or "insecurely attached" as a diagnosis — and then rebuild it as a behavioural system with a strategy. The class should leave able to say what an attachment classification is a classification of, which is a strategy under stress, not a relationship's worth.

The sentence the lecture must land. "Attachment is a strategy, not a verdict." Everything else follows: the patterns are adaptations to what a caregiver can offer; they are changeable; and the clinical task is to understand the strategy rather than to grade the relationship.

3.2 Core concepts

The behavioural system. Bowlby's term for the set of behaviours — crying, following, clinging, checking — that a child organises in order to maintain proximity to a caregiver under threat. The system is activated by danger, fatigue, illness and unfamiliarity, and it is deactivated by the caregiver's availability. It is not a feeling and not a personality trait; it is an organised set of moves that a child makes when frightened, and the organisation is what the classification describes.

The internal working model. Bowlby's second concept: from repeated experiences, the child builds a working model of what happens when I am frightened, who comes, whether I am worth coming for, and what I must do to maximise the chance of being helped. The model is not conscious and not verbal; it is a set of expectations that shapes behaviour, and it is what transfers into adult relationships.

The Strange Situation. Ainsworth's twenty-minute laboratory procedure: the infant and caregiver are in a room with toys; a stranger enters; the caregiver leaves; the caregiver returns; the caregiver leaves again; the infant is alone; the stranger returns; the caregiver returns. The classification is made on the infant's behaviour at the two reunions, not on

the separation distress — which is the detail most often missed. What matters is how the child uses the caregiver on return.

Secure (B). The infant is distressed by separation, greets the caregiver on return, is comforted, and returns to play. The strategy: my signals work, so I use them, and I can go back to exploring. The caregiver's contribution: responsive, available, and effective when the child signals.

Insecure-avoidant (A). The infant shows little distress at separation and avoids or ignores the caregiver on return, while physiological measures often show arousal. The strategy: my signals do not bring comfort, often bring rejection, so I will not signal and will manage alone. The caregiver's contribution: frequently rejecting of the child's attachment behaviour, particularly when the child is distressed.

Insecure-resistant or ambivalent (C). The infant is highly distressed, and on return is not easily comforted; may seek contact and then push away. The strategy: my signals bring inconsistent responses, so I must amplify them and cannot settle, because settling would end the attempt. The caregiver's contribution: inconsistent availability — sometimes responsive, sometimes not, in a way the infant cannot predict.

Disorganised (D). Main and Solomon's addition: the infant shows contradictory, incomplete or undirected behaviours at reunion — freezing, stilling, approaching and then turning away, a hand to the mouth. The strategy: there is no strategy, because the source of comfort is also the source of fear. The caregiver's contribution: frightening or frightened caregiving — the parent's own fear, trauma or violence enters the interaction. This is a red flag rather than a category, and it demands assessment.

The distribution, and what it means. In low-risk community samples, roughly sixty per cent secure, with the remainder divided among the insecure categories and a small proportion disorganised. In clinical and high-risk samples the proportions shift substantially. The distribution matters because it protects against two errors: the error of treating a secure classification as an achievement, and the error of treating an insecure one as a pathology.

Continuity and change. Attachment classifications are moderately stable within the first years and less stable across longer periods; they change with the caregiving environment. A child in an unpredictable household can move from secure to insecure; a child moved to a stable placement can move the other way. The clinical implication is the opposite of the popular one: attachment is not destiny, and the practitioner's task is to change the environment, not to accept the classification.

Synchrony. The matching of interaction between caregiver and infant — gaze, vocalisation, touch, pace, timing. Synchrony is not perfect imitation; it is contingent responsiveness, a caregiver responding to what the infant actually did, at a pace the infant can follow. Its absence is the earliest observable sign of relational difficulty.

The still-face experiment. Tronick's procedure: the mother interacts normally, then holds a still face for two minutes, then resumes. The infant's response is a sequence: attempts to engage, then distress, then withdrawal, then — on the mother's return —

recovery. The experiment is the strongest available demonstration of two things: that infants are active participants in interaction, and that repair after rupture is what matters, because the infant recovers rather than concluding that the relationship is broken.

Rupture and repair. Healthy development does not require constant attunement — no caregiver achieves it — but frequent enough attunement and effective repair. The concept is the single most useful in this week for family therapy, because it applies at every age: the goal is not the absence of rupture but the capacity to repair, and every family the therapist sees is organised around an unrepaired rupture somewhere.

Co-regulation. The borrowed regulation of early infancy: the caregiver's voice, body, rhythm and face down- regulate the infant's arousal, repeatedly, and the infant gradually internalises the pattern. Self-regulation is therefore a relational achievement before it is an individual capacity, which is why the advice to "let him cry it out" is a technique imposed on a relationship rather than a developmental fact.

What family therapists do with this. Three uses. First, an assessment frame: what strategy has this child developed, in this household, and what does it protect them from? Second, a formulation of the parents' own strategies: their working models are in the room, and their responses to the child's distress are shaped by what happened to them. Third, a target: the repair, not the classification — one reliably repaired rupture is worth more than a theory of the child's pattern.

3.2a Four infants, described and classified

The classification skill, practised on four descriptions, with the reasoning made explicit. Students should classify each before reading the analysis.

Infant one. Eleven months. On separation, cries briefly and looks at the door. On reunion, reaches up, is picked up, settles within thirty seconds, and then struggles to be put down so that she can return to the toys. She shows the stranger a toy from the safety of her mother's lap.

Classification: secure. Reasoning: distress at separation, active greeting, effective use of the caregiver for comfort, and a return to exploration. The critical feature is not the distress but the use of the caregiver.

Infant two. Eleven months. On separation, continues to play and does not look at the door. On reunion, does not approach, turns away, and continues with the toys; when the caregiver picks him up, he arches and looks over her shoulder. The observer notes that his heart rate rose during the separation.

Classification: insecure-avoidant. Reasoning: the absence of signalling, the avoidance at reunion, and the physiological arousal that the behaviour conceals. The strategy is the point: not signalling is a strategy, learned from a caregiver for whom signalling has not worked.

Infant three. Eleven months. On separation, is inconsolable and cannot be distracted by the stranger. On reunion, clings, cries, is picked up, and then pushes against the caregiver while continuing to cry; is not settled within the twenty minutes.

Classification: insecure-resistant (ambivalent). Reasoning: the amplified signalling, the failure to be comforted, and the mixture of contact-seeking and contact-refusing. The strategy is to keep the attempt alive, because settling would mean that the availability is settled, and the infant's experience is that it is not.

Infant four. Eleven months. On separation, stands still and does not cry. On reunion, approaches the caregiver with his head turned away, raises a hand to his mouth, and then freezes for several seconds before walking in a circle. Twice he begins to approach and stops.

Classification: disorganised. Reasoning: contradictory, incomplete and undirected behaviour; freezing; no coherent strategy. The clinical action is not a classification but an assessment: what is happening in the household, and is this infant safe?

What the four descriptions teach. That the classification depends on the reunion, not the separation; that behaviour and physiology can disagree, which is why observation alone can mislead; that every classification is a description of a strategy; and that the fourth requires an action rather than a label.

The common error. Trainees classify infant two as secure because he is not distressed. The error comes from watching the separation. The correction is simple: watch the reunion, and ask what the child does with the caregiver when the caregiver comes back.

3.3 Synchrony, assessed in the room

A family therapist does not run a Strange Situation, and can assess the same processes in an ordinary session. Six observable markers, with the behaviour that indicates each.

Marker one — the greeting. When the caregiver returns from a brief absence (leaving the room to take a call), does the infant look up, vocalise, move toward? Does the caregiver acknowledge the infant on return? A pair who do neither, repeatedly, are telling the therapist something.

Marker two — the pace. Does the caregiver wait for the infant's response before speaking again, or fill every silence? Rapid, uninterrupted caregiver speech is the most common synchrony problem in anxious parents, and it is also the easiest to change.

Marker three — the repair. When the infant becomes distressed in the session — a bump, a fright, a stranger's approach — what does the caregiver do, and how quickly does the infant settle? The repair is the assessment, and it takes ninety seconds to observe.

Marker four — the use of the caregiver as a base. Does the infant explore the room, return to the caregiver, and go out again? The pattern of excursions and returns is the secure base in miniature.

Marker five — the caregiver's reading. Does the caregiver interpret the infant's signals accurately? "He is tired" when the infant is hungry is a small error; a caregiver who cannot read any signal is a different finding. Ask the caregiver to narrate: "what is he telling you now?" The narration is the assessment.

Marker six — the caregiver's own state. A depressed, frightened, exhausted or intoxicated caregiver cannot provide synchrony, and the observation should be recorded as a statement about the caregiver's state rather than about their character. This is the marker that most often produces the referral.

How the six are used. Observed in the first fifteen minutes of a session, recorded in behavioural language, and used as the basis for the formulation. The therapist does not score the pair; the therapist describes, and the description is the intervention, because it teaches the caregiver to look at what they are already doing.

The caution. Fifteen minutes in an unfamiliar room, with a stranger present, is not a representative sample. The observation is a hypothesis generator, not a finding, and the therapist should say so and ask the caregiver what the infant is like at home.

3.3a Co-regulation, designed for one family

The concept is easy to state and hard to apply. A worked sequence for a family in which the caregiver's own regulation is the problem.

The family. A mother, 24, with a nine-month-old, referred by the health visitor for "he cries all the time and I cannot cope". She is sleeping four hours a night, has no help, and describes herself as "shouting at a baby". Her own history includes a violent father and a mother who "never held us".

The formulation. The infant's crying activates the mother's own attachment system: she experiences the crying as an accusation, her arousal rises, and she responds with the only strategy she has — withdrawal, followed by shouting. The infant's crying escalates, the mother's arousal escalates, and the loop is complete within ninety seconds. Neither is doing anything pathological; both are doing what their systems have learned.

The co-regulation sequence, taught in the session.

Step one — the mother's own regulation, first. "Before you can calm him, you have to lower your own arousal, because he reads you. What is the fastest way for you to bring yourself down?" Her answer: cold water on her wrists, and counting to ten out loud. Both are taught and rehearsed in the room, with the baby present, so that she experiences the sequence rather than being told about it.

Step two — a holding position that works for her. Not the textbook position: a position she can sustain without feeling trapped. She chooses to sit on the floor with him against her chest, which is a position she can leave if she needs to, and which she can hold for two minutes.

Step three — a sound she can make. A low, repeated sound, not a song, because a song requires performance. She chooses "shhh" on the out-breath, which is also the physiological sigh that lowers her own arousal, so the technique regulates both of them at once.

Step four — the two-minute rule. She commits to two minutes of steps one to three before doing anything else. Two minutes is chosen because it is achievable when she is exhausted, and because most infant crying cycles change within two minutes.

Step five — the permission to put him down. "If after two minutes you still cannot, put him in the cot, leave the room, and stand at the door for three minutes. That is not abandonment; that is you preventing something worse." The permission is the intervention's ethical core: a mother who is frightened of her own anger needs a legitimate exit.

The follow-up. Two weeks later: the two-minute sequence is used "most times", the shouting has reduced, and the mother reports one occasion on which she put him down and stood at the door. She describes it as "the first time I did not feel like a bad mother". The infant's crying has not disappeared; the loop has been interrupted.

What the sequence demonstrates. Co-regulation is not a technique for the infant. It is a technique for the relationship, and it begins with the caregiver's own state — which is why this week's material belongs in a family therapy course and not only in a developmental psychology text.

3.3b The caregiving network, mapped

The single most useful correction to the dyadic literature, and the instrument that puts it into practice.

The instrument. A circle in the middle for the infant, and around it every person who does caregiving work, with three pieces of information: how often, for how long, and what they do. The answers are usually wider than the referral: a grandmother who takes the nights, an aunt who takes the child at weekends, an older sibling who walks him to nursery, a neighbour who minds him while the mother sells.

Why it matters clinically. Three reasons. First, the assessment of "the mother–infant relationship" is incomplete and often unfair when four people are raising the child; the mother is graded on a job she is doing with help, or failing to do because the help is contested. Second, the infant's attachment strategies are distributed across the network: a child may be secure with the aunt and avoidant with the father, and the classification literature's single-figure procedure cannot describe this. Third, the network is where the intervention usually belongs: the contested rule, the inconsistent method, the caregiver who undermines another, the person who is exhausted.

The three questions that map it. "Who has held him this week, and for how long?" "Who decides what happens when he cries?" "Who disagrees with whom about how he should be raised?" The third question is the clinical one, and it is almost never asked in a developmental assessment, although it explains more of the household's difficulty than any milestone.

A worked map. The Balogun infant of Week 2: mother (all nights, most days), father (rest days, evenings), paternal grandmother (four nights a week, all cooking, some feeds), an aunt (Saturdays), and a neighbour (two afternoons a week while the mother works). Decisions: the grandmother decides feeding and bathing; the mother decides sleep; nobody decides anything about the crying, which is where the disputes are. Disagreement: the grandmother believes the infant should be left to cry; the mother cannot bear it; the father follows whoever is in the room.

The intervention the map produces. Not a parenting programme. One conversation with the three adults about crying — what they each do, what they each believe, and one agreement they can all keep — plus the assignment of one decision to the mother, explicitly, so that she has an area of authority. The map made the problem visible: the household was not failing to respond; it was responding four different ways.

The caution. Mapping the network can expose a conflict that the family have managed by not discussing it, and the therapist should not conduct the exposure in a first session with the infant present. Map it, note it, and bring the adults together separately when the map is complete.

3.4 The first session with an infant and two parents

An assessment structure for the youngest client, in five phases, with the observation built into the session rather than bolted on.

Phase one — the frame (five minutes). "I am going to ask about the pregnancy, the birth and the first months, and then I will spend some time just watching the three of you together, because that tells me more than any question. You do not need to perform anything; do what you would normally do." The permission not to perform is necessary, because parents of infants believe they are being graded.

Phase two — the chronology (fifteen minutes). The pregnancy, the birth, the first weeks, feeding, sleep, who has helped, and what has changed in the household. The timeline is drawn as in Week 1, with one addition: the parents' own events — a job loss, a bereavement, a move — are marked on the same line as the infant's, because the two are one story.

Phase three — the observation (fifteen minutes). The therapist stops asking questions and watches. The six markers of 3.3 are noted silently: greeting, pace, repair, use of the caregiver as a base, the caregiver's reading, the caregiver's own state. The therapist says nothing for the first five minutes, which is uncomfortable for everyone and is the most informative part of the session.

Phase four — the narration (ten minutes). "Can I tell you what I noticed? You waited for him to look at you before you picked him up, and when the door banged he looked at you before he cried. That is a child who checks with you." The narration is in behavioural language, it names what is going well first, and it is specific. The parents' response to the narration — relief, tears, argument — is itself data.

Phase five — the formulation (five minutes). One of the five statements from Week 1, plus one thing to try before the next session. The thing to try is almost always relational and small: a two-minute co-regulation sequence, a pause before responding, a handover of one caregiving task between the parents.

What the therapist does not do. Does not test the infant; does not classify the attachment; does not interpret the parents' childhoods in session one; does not give feeding or sleep advice unless asked, and then with the evidence stated plainly in both directions.

3.4a The still face, in the consulting room

The experiment cannot be replicated with a family, and its lesson can be demonstrated, and the demonstration is the week's most powerful teaching device.

The demonstration. With a volunteer parent and infant (or a peer and a doll), the therapist asks the parent to play normally for one minute, then to hold a still face for thirty seconds, then to resume. Most parents find the thirty seconds distressing, and most infants mount a visible sequence of attempts, protest, and withdrawal within twenty seconds. The debrief is the teaching.

What the demonstration shows. That the infant is not a passive recipient but an active partner with strategies; that the infant's distress is proportionate and immediate; that the resumption produces recovery in most pairs; and that the parent's own discomfort during the still face is a reliable index of how much the interaction matters to them.

The clinical use. The demonstration is used with parents who believe their infant does not notice them — fathers who feel irrelevant, mothers who believe the baby prefers the grandmother, parents who have been told that "at this age they do not know anything". Showing a parent that their infant is tracking them is more persuasive than any amount of explanation, and it changes the parent's behaviour within the session.

The caution. Never with a distressed or ill infant, never without consent, never for longer than thirty seconds, and never as a test of the parent. The demonstration's purpose is the parent's discovery, not the therapist's evidence.

3.5 Case: the Adeyemi family, in the infant's terms

The course's recurring family, now read for the infancy that shaped it — because the infancy is where the patterns the family present were built.

What we know. Amara is fourteen; Segun is eleven. Their father, Tunde, has worked away for most of their childhood. Their mother, Funke, has managed the household alone. The paternal grandmother lived with them until her death in 2023.

The reconstruction, offered as a hypothesis and not as a finding. Funke's account: Tunde was present for the first three months of Amara's life, then took the contract in Lagos. She describes those three months as "the best time" and the following year as "the worst". She returned to work at six months and left Amara with her mother-in-law, with whom she "did not agree about anything but could not say so".

The attachment reading. Amara's first year contained two discontinuities: her father's departure at three months, and her mother's return to work at six with a substitute caregiver whose methods her mother could not endorse. Neither is a trauma; both are the ordinary material from which a working model is built. The hypothesis — and it is a hypothesis — is that Amara learned that the person who is most available is also the person whose authority is contested, and that need is best managed by being self-sufficient and by keeping the adults' attention through achievement and, later, through refusal.

Segun's first year, by contrast. Funke did not return to work until Segun was two, and the grandmother was already ill, so Funke was at home. Segun's strategy is different: he stays close, he manages the household's information, and he is the family's listener. The difference between the siblings is a natural experiment in caregiving conditions, and the family's own account confirms it.

What the therapist does with the reconstruction. Not much, and carefully. The reconstruction is used for two purposes: to understand why the mother's pursuit and the daughter's withdrawal are so well rehearsed, and to ask one question that opens the history without demanding it — "when Amara was small, who held her?" The question is asked once, in the presence of both parents, and whatever comes back is received.

The caution. Retrospective reconstruction of infancy is a hypothesis, not a record; memory is unreliable and the family's account is shaped by the present. The therapist who presents a reconstruction as a finding has replaced the family's story with a theory, which is the error Week 11 of MFT 702 warns against. The reconstruction earns its place only if it produces a question the family can use.

3.6 Annotated transcript: rupture and repair in a session

Ten turns from a first session with a mother (26) and her eleven-month-old, referred for "he will not settle with anyone but me". The transcript shows a repair happening live.

> **T1 Therapist:** He is looking at the box of toys. What does he usually do in a new room?
> **T2 Mother:** He clings. He will not go.
> **T3 Therapist:** (waits) He has let go of your sleeve.
> **T4 Mother:** (looking) He has.
> **T5 Therapist:** What would happen if you sat on the floor and did not reach for him?
> **T6 Mother:** He will cry.
> **T7 Therapist:** Let us find out. I will be quiet.
> **T8 Mother:** (sits on the floor; the infant moves two steps, looks back, and she reaches for him) — see?
> **T9 Therapist:** He looked back first. That is what he does before he decides. If you wait for the second look, he might go further. Would you try it once more?

> **T10 Mother:** (sits still; the infant moves four steps, looks back twice, she stays; he brings a block back to her) — he brought it to me.

Annotations. T1 begins with the child rather than the complaint, and asks the mother for her prediction, which is the assessment of her reading.

T2 is the maternal expectation — "he clings" — which is both accurate about the past and a prediction that will organise the future. The therapist does not challenge it; she waits.

T3 is the noticing, in behavioural language, of something the mother has not seen because she is answering a question. The therapist's silence at the start of the turn is doing work: it gives the infant time to act.

T5 is the invitation, framed as an experiment rather than an instruction, which is what makes it acceptable. T7 explicitly removes the therapist from the interaction, which lowers the pressure and makes the mother the agent.

T8 is the rupture: the mother reaches for the infant at the first look, which is precisely the pattern that prevents exploration. Her "see?" is not defiance; it is the proof of her own prediction.

T9 is the repair, and it is the transcript's most skilful move. The therapist does not say "you are over-responsive"; she names the mechanism ("he looked back first; that is what he does before he decides") and offers a small modification ("wait for the second look"). The mother's prediction is not contradicted; it is given a condition.

T10 is the outcome, and it is the mother's sentence — "he brought it to me" — not the therapist's. The intervention was ninety seconds long, and it changed a working model in the room.

What the trainee should notice. The therapist made no interpretation about the mother's own childhood, gave no advice about independence, and did not celebrate the outcome. The technique is a wait, a noticing, and an invitation, and it requires the therapist to tolerate the mother's distress at T8 without rescuing either of them.

3.7 Laboratory: observing synchrony

Sixty minutes, in threes, with a video clip and then a live observation.

Task one — the clip (twenty minutes). A consented three-minute clip of a caregiver and infant. First five minutes: description only, in behavioural language, no interpretation. Then the six markers, noted. Then one rupture and one repair, described.

Task two — the live observation (twenty-five minutes). One student brings a recording (or plays a parent with a doll); the therapist observes for five minutes in silence and then narrates what they saw, beginning with what is going well. The observer checks whether the narration was behavioural and whether it named a strength first.

Task three — the co-regulation design (fifteen minutes). For the family in 3.3a, design the five-step sequence and rehearse step one out loud.

Marking rubric.

Criterion	Weight	Full marks
Behavioural description	25%	No interpretation for the first five minutes; behaviour, timing and sequence named
Marker identification	20%	All six addressed, including the caregiver's own state
Rupture and repair	20%	One of each identified and described in behavioural language
Narration	20%	Strength named first; specific; the parent could repeat it
Co-regulation design	15%	Begins with the caregiver's regulation; achievable; an exit included

The debrief question. "What did you want to interpret, and what stopped you?" The answers are usually about the parents' own histories, and the discussion of why a first session is not the place for that interpretation is the week's most useful ten minutes. The rule to take away: describe before you explain, and explain only what the family have already begun to say.

3.8 Six errors in infant mental health work

1. **The label as a finding.** "He is insecurely attached" said to a parent who came for help with sleep. Fix: describe the strategy, not the classification, and only where it serves the work.
2. **The mother as the patient.** The observation focused entirely on the mother, with the father, the grandmother and the household invisible. Fix: the caregiving network as the unit, and every caregiver observed.
3. **The interpretive leap.** "He clings because you are anxious" offered in the first session, before trust. Fix: describe, narrate, then hypothesise — in that order, over sessions.
4. **The technique imposed.** Sleep training, independent play, or "let him cry" prescribed without attention to the family's capacity, beliefs and circumstances. Fix: the evidence stated plainly in both directions, and the family's decision respected.
5. **The missed caregiver state.** A depressed, frightened or exhausted caregiver observed and not addressed, because the referral was about the baby. Fix: the caregiver's own state is the sixth marker, asked about and responded to.
6. **The ignored history.** A disorganised pattern observed and not followed up, because following it up would require asking about violence. Fix: ask, record, and act — the same screen as Week 2, applied to infancy.

3.9 Culture, faith and the first relationship

Where the week's material travels well. The behavioural-system account is one of developmental psychology's more portable ideas: every human infant signals, every caregiver responds or does not, and the resulting strategies are recognisable across cultures. The concepts of synchrony, rupture and repair are observable in any household, and the co-regulation sequence is a description of what most caregivers already do.

Where it strains, four ways.

The Strange Situation's assumption of the dyad. The procedure assumes a single primary caregiver, and it assumes that the infant's reaction to a stranger and to a brief separation is meaningful in the same way everywhere. In households where multiple caregivers are routine — grandmothers, aunts, older siblings, paid helpers — the infant's reaction to the departure of one figure is a different measurement. The literature on multiple attachments is clear that infants form several, and the clinical assessment should therefore map the network rather than grade the mother.

The value placed on exploration. The secure pattern's marker is a return to play, which encodes a cultural valuation of exploration and independence. A household that values

compliance and closeness will produce an infant who stays near the caregiver, and the observer who scores exploration as the healthy outcome has imported a value into the assessment. The clinical question is not "does he explore?" but "does he use his caregiver when he is frightened?"

The practice of separation. Some households do not leave an infant with a stranger, ever, and some leave the infant for months with a relative in another town. Both produce patterns the laboratory did not anticipate. The therapist's task is to understand what the separation meant in this family, and to ask about it rather than to infer from a classification.

The independence curriculum. Sleep training, feeding schedules and early independence are cultural practices presented in some of the literature as developmental norms. A family who sleep with their infant, feed on demand and carry the child are not failing a developmental task; they are following a different curriculum, and the therapist who treats the curriculum as the problem has mistaken a practice for a milestone.

Faith and the first relationship. Naming ceremonies, blessings, dedication, prayer over a newborn and the forty-day lying-in period are all attachment-relevant events: they mark the child's entry into a community that will share the caregiving. The therapist should ask who was present, what was done, and who has a claim on the child's care as a result. The answers frequently reveal a caregiving network that the referral had rendered invisible, and in some families they reveal a dispute about the child that is the real presenting problem.

The examinable sentence. "Attachment is a strategy learned in a particular caregiving network, and the clinical task is to describe the strategy and the network, not to grade the pair."

> **DEBATE**
> "The Strange Situation measures a relationship, and using it to classify a child is a category error." Argue both sides, and say what the classification is useful for.

3.10 Evidence note: strong theory, moderate evidence, popular misuse

What is well established. That infants organise attachment behaviour and that individual differences can be reliably classified; that maternal sensitivity predicts security, with a moderate association; that classification is moderately stable in the early years; that disorganised classification is associated with frightening or frightened caregiving and with later difficulties; and that early security predicts later social functioning, weakly to moderately, with the effect mediated by later experience.

What is contested or weaker than commonly claimed. The strength and specificity of the link between infant attachment and adult attachment style — the transmission literature is real and the effect sizes are modest; the predictive power of attachment classification for later psychopathology, which is small in low-risk samples; the claim that the first two years determine later relationships, which the evidence does not support;

and the clinical usefulness of attachment classifications in ordinary family work, as against the description of strategies.

What the popular literature gets wrong. Three things, all of which families bring into the consulting room. That attachment is love, so that a "securely attached" child is a loved child and an "insecurely attached" one is not. That attachment is fixed, so that a difficult first year is a life sentence. And that attachment is a parenting technique — baby-wearing, co-sleeping, feeding — rather than a property of the relationship's reliability. Each of the three is a distortion, and each is common enough that the practitioner should be able to correct it in one sentence.

What the practitioner may claim. "Infants develop strategies for getting help from the people who care for them; those strategies are describable; they are shaped by the caregiving they receive; they change when the caregiving changes; and the repair of ruptures matters more than the avoidance of them." Each claim is defensible, and together they are enough for clinical work.

3.11 Key terms

Term	Definition
Attachment behavioural system	The organised behaviours a child uses to maintain proximity to a caregiver under threat
Internal working model	The child's expectations about self, others and help, built from repeated experience
Strange Situation	Ainsworth's laboratory procedure, classified on reunion behaviour
Secure (B)	Signals used, caregiver effective, exploration resumed
Avoidant (A)	Signalling suppressed, caregiver avoided at reunion, arousal often elevated
Resistant/ambivalent (C)	Signalling amplified, contact sought and refused, difficulty settling
Disorganised (D)	Contradictory, incomplete or undirected behaviour; no coherent strategy
Synchrony	Contingent, paced responsiveness between caregiver and infant
Still face	Tronick's procedure demonstrating infant participation and the importance of repair
Rupture and repair	The ordinary cycle of mismatch and restoration that builds resilience
Co-regulation	The borrowed regulation of arousal that precedes self-regulation
Caregiving network	All those who care for the infant, not only the parents

3.12 Self-test

1. What is an attachment classification a classification of?
2. Why does the reunion, rather than the separation, determine the classification?
3. Describe the strategy of the avoidant infant and the caregiving that produces it.
4. What makes disorganised attachment a red flag rather than a category?
5. State the relationship between synchrony, rupture and repair.

6. What does the still-face experiment demonstrate, and how is it used clinically?
7. Why is co-regulation a relational achievement?
8. Name three ways the popular literature misuses attachment, and correct each in one sentence.

Answers. 1. Of the strategy a child has developed for obtaining help from a caregiver under stress. 2. Because the strategy is revealed in what the child does with the caregiver when the caregiver returns — the separation shows distress, the reunion shows the organisation. 3. The avoidant infant suppresses signalling and avoids the caregiver, while physiology often shows arousal; the caregiving is typically rejecting of attachment behaviour, particularly distress. 4. Because it indicates contradictory or absent strategy, is associated with frightening or frightened caregiving, and requires an assessment of safety rather than a label. 5. Synchrony is frequent enough attunement; ruptures are inevitable; repair after rupture is what builds the child's confidence that the relationship survives. 6. That infants are active participants who respond to unresponsiveness and recover when responsiveness resumes; clinically it shows parents that their infant is tracking them. 7. Because the capacity to calm is first borrowed from a caregiver's voice, body and rhythm, and only later internalised.

1. That it is love (it is a strategy); that it is fixed (it changes with caregiving); that it is a technique

(it is a property of the relationship's reliability).

3.12a What to tell a parent in one page

The clinical material of this week, converted into something a parent can take home. Six sentences, in ordinary language, each with the developmental idea behind it.

1. **"He is not manipulating you; he is asking you for something, and he only has one way of asking."** The behavioural system is a communication system, and the infant's signals are the whole of the vocabulary.
2. **"You do not have to get it right every time. You have to get it right often enough, and come back after the times you miss."** Rupture and repair: the repair is what the infant is learning about, not the perfect attunement.
3. **"When you calm him, you are lending him your calm. Later he will do it himself, because he has borrowed it from you a thousand times."** Co-regulation as the precursor of self-regulation.
4. **"He checks your face before he decides how to feel about things, so your face is part of how he learns to feel."** Social referencing, and the reason a parent's own state is part of the child's environment.
5. **"If you are frightened, he will be frightened, and that is not your fault; it is the reason we are working on your situation as well as his."** The sixth marker, and the reason the assessment is of the household.

6. **"There is no single right way to raise a child in this family; there is the way you and the people who help you can agree on, and the agreement is what he needs."** The caregiving network, and the reason the therapist maps it.

How the page is used. Read aloud in the session, in the family's language, with the parent's own examples substituted for the general statements. Handed over at the end, with the therapist's name and the next appointment — not as a leaflet, but as a record of the conversation the family has just had.

3.13 Reflective task

Watch a caregiver and an infant for fifteen minutes in a public place — a bus, a clinic waiting room, a market. Describe, in writing, three things: one moment of attunement, one rupture, and one repair if there was one. Do not interpret, and do not judge. Then write two sentences on what it felt like to watch without interpreting. Most students find the third instruction the hardest, and the discomfort is the week's lesson.

3.14 Readings for this week

- **Bowlby (1969)**, Attachment and Loss, Vol. 1, the founding statement, read the chapters on the behavioural system.
- **Ainsworth et al. (1978)**, Patterns of Attachment, for the Strange Situation and the caregiving correlates.
- **Tronick (2007)**, The Neurobehavioral and Social-Emotional Development of Infants, for the still face and the repair literature.
- **Sroufe (1996)**, Emotional Development, for the longitudinal account of continuity and change.
- **Main & Solomon (1990)**, on disorganised attachment, read with the caution the concept requires.
- **Keller (2007)**, Cultures of Infancy, the essential corrective on the dyad and the caregiving network.

Read Keller before Ainsworth; the corrective first, so that the classification is read as a description of one kind of caregiving arrangement. For the case analysis, use at least one source that disputes the predictive power of attachment classification, and say what it means for your formulation. The reading-log question this week is: "what does attachment language let me see, and what does it stop me seeing?"

Next: Week 4 — Toddlerhood: autonomy, language and limits

3.2b The four patterns in adult life: what the family therapist sees

The infant patterns are the course's foundation for later work, and the bridge to the adult literature is worth building explicitly, because it is what the families' own patterns look like in a session.

Pattern	In the infant	In the adult, under stress	What the therapist sees in a couple
Secure	Signals, is comforted, returns to play	Seeks support, tolerates the answer, negotiates	Asks for what they need, hears the answer, can repair
Avoidant	Suppresses signalling, avoids at reunion	Manages alone, dismisses the need, works	Withdraws, gives facts, resists affect, seems calm
Resistant	Amplifies signalling, hard to settle	Escalates, monitors, cannot be reassured	Pursues, protests, cannot register the reassurance offered
Disorganised	No coherent strategy; freezes	Approach and withdrawal in the same minute	Contradictory moves, sudden coldness, intense pursuit then flight

Why the bridge is useful and dangerous. Useful because it gives the trainee a vocabulary for the couple's pattern that connects to a child's experience rather than to a personality type. Dangerous because the adult categories are not the infant classifications, and the popular habit of describing a partner as "avoidant" or "anxious" collapses a strategy under stress into an identity. The therapist's language should keep the condition: "under stress, you manage alone; under stress, you escalate" — not "you are avoidant".

The clinical use in three sentences. "When your wife raises her voice, you go quiet and give facts, and she escalates because the quiet looks like indifference. Both of you are doing what you learned to do when you were frightened, and by the time you are in this room both of you are frightened. The work is to change what happens in the ninety seconds after the fear, not to change who you are." That paragraph is the bridge, and it is recognisably Week 7 of MFT 702 in developmental clothing.

The three cautions. First, do not diagnose attachment styles from a session; the pattern you observe is a pattern under the specific stress of being observed. Second, do not use the vocabulary as a verdict — "he is an avoidant type" — because it does what the classification was designed not to do. Third, do not explain the parents' childhoods to them in the first months of therapy; the reconstruction is a hypothesis, and it is asked about, not delivered.

3.7a The disorganised flag, and what followed

A case in which the fourth pattern appeared in a session and required an action rather than a formulation.

The referral. A mother (29) and her fourteen-month-old, referred by a community nurse for "he is frightened of her". The mother describes the child as "always crying" and says she has "lost her patience". She is reluctant to come and says the nurse "made her".

The observation. In the first twenty minutes the infant plays on the floor, and twice, when his mother stands up to reach for a bag, he freezes with his hand at his mouth. When she sits again, he approaches to within a metre and stops. When she speaks sharply to the therapist, he stops playing entirely and watches his mother's face.

The therapist's internal response. A wish to reassure the mother, and a competing wish to protect the child. Both are data. The therapist notes them and does not act on either in the session.

The action, in the session. The therapist does not interpret, does not classify, and does not ask about violence in front of the infant. She ends the session ten minutes early, with a practical frame ("I would like to see you again this week, on your own, before we meet with him again"), and she records the observation in behavioural language: "infant froze twice on maternal movement; hand-to-mouth; stopped play on maternal sharp speech".

The individual session. Asked directly and privately: "is there anyone in the house who frightens you?" The mother cries and describes a partner who shouts and who has pushed her twice, "but not near the baby". The therapist does not accept the qualification as reassurance and does not argue with it. She names the risk to the infant plainly, once: "a baby who freezes when you stand up is a baby who has learned that movement can be dangerous. That is not your fault and it is not nothing."

The plan. A safety plan for mother and infant; referral to a domestic abuse service with the mother's agreement; a discussion about whether the partner should be seen, and a decision that he should not, for now; and a report to the safeguarding lead, explained to the mother in advance, in plain language, with the reason.

What happened. The partner left the household after a further incident. The mother and infant were seen for four months. The freezing at maternal movement disappeared within six weeks; the mother's own account of her history — a violent father, a mother who "never held us" — was the subject of the middle phase of the work.

What the case teaches. That the fourth pattern is a signal that requires an action, not a category to be recorded; that the observation must be written in behavioural language if it is to be used; that the screen is asked privately and the answer acted on; and that the therapist's own wish to reassure is a warning sign, because it is usually the wish to avoid what the observation has already shown.

Week 4 — Toddlerhood: autonomy, language and limits

Developmental tasks. Autonomy versus shame and doubt (Erikson); language explosion (from ~50 words at 18 months to 300+ and two-word combinations by 24 months); symbolic play; beginning of self-regulation.

The clinical translation of "no". A toddler's "no" is the emergence of a separate self. The parental task is to hold the limit without humiliating the child — "You may not hit. You may be angry." This is the earliest training ground for the emotional vocabulary the family will use for the next fifteen years.

Language and family. Delay in expressive language is the most common developmental concern presented to clinics. Distinguish: late talker (normal variation with good comprehension), bilingual language confusion (normal), hearing loss, global delay, and

autism spectrum presentation. Always ask about **hearing** and about **which language is spoken at home and by whom**.

> **CASE**
> A grandmother raises a 2-year-old while both parents work. She reports he "does not listen". Assessment finds: he is spoken to mostly in commands, in two languages, by three adults with different rules. The intervention is not a behaviour chart; it is a family agreement about one set of five rules, one language of instruction, and ten minutes of child-led play a day with each caregiver.

Week 5 — Early childhood: play, theory of mind, initiative

Tasks. Initiative versus guilt; fantasy play; theory of mind (understanding that others have different beliefs — typically emerging around 4); moral development begins as consequence-based reasoning; school readiness.

Play is the work. Play develops language, self-regulation, perspective-taking, and the capacity to tolerate frustration. Children who have experienced adversity often play less symbolically and more repetitively — a clinical observation that underpins play therapy (MFT 727).

The clinical skill: observing play without hijacking it. Describe what the child does; do not direct it, and do not interpret it prematurely ("that must be your father"). Interpretation before trust produces a performance, not a communication.

> **PRACTICAL**
> Ten minutes of child-led play with a volunteer child or peer. Your only tools: describe, reflect, imitate, and set one limit. Debrief: how many times did you ask a question? (Aim for fewer than three.)

Week 6 — Middle childhood: industry, peers, school

Tasks. Industry versus inferiority; mastery and competence; peer group and friendship rules; self-concept becomes comparative ("I am good at maths, bad at football"); moral reasoning becomes more reciprocal.

What changes in the family. The child's world widens; parents move from direct control to monitoring and scaffolding. School becomes a second system with its own rules, and the family–school interface becomes a clinical site.

School refusal — a developmental and systemic differential. Ask: is this anxiety, separation difficulty, bullying, a learning difficulty, a family crisis, or a bid to protect a parent? The distinction matters: the same behaviour has opposite treatment implications.

> **CASE**
> A nine-year-old refuses school on Mondays only. Developmental enquiry: what happens on Sunday nights? The answer: the parents' weekly argument about money. The child's presence at home on Monday keeps the mother from leaving for work, and the argument stops. The symptom is a Monday-shaped solution to a marital problem — and it is also genuinely distressing to the child.

Week 7 — Adolescence I: puberty, identity, autonomy

Tasks. Identity versus role confusion; pubertal change; separation-individuation without severance; peer belonging; emerging sexuality; moral and ideological commitment.

The neuroscience, briefly. The adolescent brain is not "broken": it is specialised. The reward system matures before the regulatory system, which produces sensation-seeking and heightened sensitivity to peer approval alongside gradually improving impulse control. This is an argument for scaffolding, not for lecturing.

The family task. To renegotiate the relationship: less control, more influence; more privacy, more accountability. Families that cannot renegotiate produce two common patterns: escalation (control increases as autonomy increases) and abdication (parents withdraw entirely).

> **CASE**
> Amara, 16. The parents describe her as "secretive"; she describes them as "spies". The developmental reframe: privacy is a developmental need, and secrecy is what privacy becomes when there is no negotiated trust. The intervention is a negotiation: what will she share, what will they ask, and what happens when the agreement is broken?

Week 8 — Adolescence II: risk, peers, digital life and mental health

Risk behaviour is normative in moderation and dangerous in combination: substance use, risky driving, unsafe sex, self-harm, disordered eating. Risk is not a moral category; it is a developmental phenomenon shaped by peers, family, school and access.

Digital life. Assess function, not just quantity: what is the phone doing in this adolescent's life — connection, escape, status, self-soothing, harm? Ask about sleep, cyberbullying, sexual content, and the family's rules about devices. Banning without substituting an alternative rarely works.

Mental health. Half of all lifetime mental disorders begin by mid-adolescence. Screen, do not guess. Distinguish transient distress from disorder by **duration, impairment and severity** — and use the same developmental question: is this a normal task being struggled with, or a disorder?

PRACTICAL

Conduct a 12-minute adolescent interview with a peer who has been briefed to be reticent. You may not ask more than two questions in a row without reflecting something back. Observer records: ratio of questions to reflections; did the adolescent elaborate?

Week 9 — Adult development

Tasks. Intimacy versus isolation; career and work identity; generativity versus stagnation; the "social clock" (the culturally shared timetable for marriage, children, promotion, retirement).

Why this matters clinically. Most couple problems presented in clinics are adult developmental problems wearing a relational mask: a 32-year-old's struggle with intimacy, a 45-year-old's reckoning with a career, a 58-year-old's grief for the life they did not have.

Life structure and the "seasons" of adulthood (Levinson; vaillant's longitudinal work). Treat these as metaphors with empirical support, not as laws. The strongest empirical finding is **continuity with change**: people change, but in patterned ways that reflect early relationships and current context.

CASE

A 45-year-old man presents with "irritability". Developmental enquiry finds two things: his father died at 45, and his eldest child is about to leave for university. He is not depressed in the clinical sense; he is renegotiating mortality and role. The intervention is narrative and relational — mourning the father he barely knew, and planning what his own next decade is for.

Week 10 — Couple development

Couples develop too. A workable map (used again in MFT 710 and 718):

Phase	Central task	Common failure
Formation	Commitment and identity as a couple	One partner never really chooses
Early marriage	Building rituals and boundaries with families of origin	In-law triangles
Young children	Dividing labour; protecting the couple bond	Co-parenting replaces partnering
Adolescent children	Re-negotiating autonomy, revisiting one's own adolescence	Escalation or abdication
Launching	Re-intimacy after children leave	The "empty nest" that was already empty
Later life	Facing illness, retirement, loss together	Caretaking replaces relating

> **PRACTICAL**
> With a peer, draw a couple timeline: five high points, five low points, one turning point each. Then ask: which developmental transition explains the lowest point? Present in 5 minutes.

Week 11 — Parenting across stages and talking to parents

Parents do not need lectures; they need **accurate, specific, hopeful information** in plain language. The skill has four parts:

1. **Normalise first.** "What you are describing is very common at this age."
2. **Give one developmental fact.** "At four, the part of the brain that manages impulse is still being built, so he cannot yet do what you are asking consistently."
3. **Give one concrete action.** "Try naming the feeling and offering two acceptable choices."
4. **Set a review.** "If this is still happening in three weeks, or if X or Y appears, come back."

What to avoid: technical vocabulary; predictions ("he will grow out of it"); blame; over-reassurance when the concern is real; and overwhelming a parent with five recommendations.

> **PRACTICAL (OBSERVED)**
> Ten-minute parent-feedback role-play using the four-part structure. Assessor rates: plain language, one fact, one action, review point, and warmth.

Week 12 — Later life

Tasks. Integrity versus despair; adaptation to retirement; bereavement; caregiving; grandparenthood; legacy and meaning.

Clinical realities. Cognitive change must be distinguished from delirium, depression and dementia; hearing loss is a major and under-treated contributor to isolation and apparent confusion. Caregiving burden falls disproportionately on women and on the "responsible" child, and it frequently reopens old sibling hierarchies.

Grief. Use the dual-process model (Figure 22, MFT 712): healthy grieving oscillates between loss-oriented and restoration-oriented activity. In many cultures, mourning is collective and prescribed; the clinician's task is to support the rites rather than to impose a private model of "working through".

> **CASE**
> Three siblings disagree about their mother's care. The eldest, a daughter, has done everything; the youngest, a son abroad, sends money and criticises. The clinical task is not to decide who is right but to make the loyalty ledger explicit: who owes what, to whom, and by what accounting? (This is contextual therapy's territory; see MFT 702 and 715.)

Week 13 — Culture, poverty and development

Development is not culture-free. Three critiques you must be able to state:

1. **The norms are Western.** Attachment classifications, individuation timetables and "school readiness" were normed on particular populations. A child who sleeps in the parental bed until five is not necessarily enmeshed; in many cultures this is ordinary.
2. **Poverty changes development.** Chronic stress, food insecurity, crowding and insecure housing affect attention, sleep, and the capacity for consistent parenting. Diagnosing "inconsistent parenting" without naming material conditions is a category error.
3. **Extended and distributed caregiving is normal.** In much of the world, children are raised by networks — grandparents, aunts, older siblings, co-wives, foster kin. Attachment theory travels better when it is asked "who does this child seek when frightened?" rather than "is the mother attuned?"

> **CRITICAL READING**
> Read one cross-cultural critique of attachment theory and one study of caregiving in a non-Western setting. Write 500 words on what you would change in your own assessment template as a result.

Week 14 — When development goes wrong

Build a differential habit. For any concern, run the four-category check (variation, delay, regression, symptom) and then ask the five systemic questions:

1. **When did it start, and what else changed then?**
2. **Who noticed first, and who disagrees?**
3. **What does the family do when it happens, and what happens next?**
4. **What would improve if it stopped — and what would get worse?**
5. **What has already been tried, and what did that teach us?**

Presentation	Developmental differential	Systemic differential
Aggression at 5	Language delay; impulsivity; trauma	Parental conflict; inconsistent limits; a new baby

Presentation	Developmental differential	Systemic differential
Withdrawal at 12	Depression; bullying; neurodivergence	Family secrecy; a parent's illness; migration
Decline at 70	Delirium; dementia; depression; hearing loss	Bereavement; relocation; loss of role
"Not listening" at 3	Hearing; language; attention	Multiple caregivers, conflicting rules

Week 15 — Integration

The developmental lens does three things for your clinical work: it **calibrates** your expectations, it **dates** the problem (onset relative to transitions), and it **predicts** what the family must renegotiate next. Carry three questions into every first session: What is this person's developmental task right now? What is the family's corresponding task? And what would "on track" look like in six months?

Glossary

Attachment — a behavioural system organising proximity-seeking to a caregiver under threat. **Co-regulation** — the process by which a caregiver's calm and responsiveness regulate the child's arousal. **Developmental task** — the psychosocial challenge characteristic of a life stage. **Disequilibrium** — a period of developmental reorganisation marked by increased conflict or regression. **Goodness of fit** — the match between a child's temperament and the demands of the environment. **Regression** — loss of previously acquired function; a red flag. **Scaffolding** — temporary support that allows a child to do what they cannot yet do alone. **Separation-individuation** — the process of developing autonomy while remaining connected. **Social clock** — culturally shared expectations about the timing of life events. **Theory of mind** — the capacity to attribute mental states to others. **Zone of proximal development** — the gap between what a learner can do alone and with support.

Self-assessment

Multiple choice

1. A four-year-old who stops speaking after a family bereavement is best described as: (a) normal variation (b) delay (c) regression (d) attention-seeking

2. "Goodness of fit" refers to the match between: (a) child and school (b) temperament and environment (c) parents' goals (d) IQ and achievement

3. The most common developmental concern presented to clinics is: (a) motor delay (b) expressive language delay (c) sleep (d) feeding

4. Disorganised attachment is most associated with: (a) daycare (b) frightening or frightened caregiving (c) prematurity (d) bilingualism

5. A toddler's frequent "no" primarily reflects: (a) oppositional disorder (b) emerging autonomy (c) poor parenting (d) language delay

6. Which is not a feature of adolescent brain development? (a) heightened reward sensitivity (b) peer sensitivity (c) mature impulse control (d) later prefrontal maturation

7. A "social clock" refers to: (a) sleep rhythms (b) culturally shared timing of life events (c) school timetable (d) biological ageing

8. In the dual-process model, healthy grieving involves: (a) staying on the loss track (b) avoiding reminders (c) oscillating between loss and restoration (d) suppressing emotion

9. Apparent confusion in an older adult should first prompt assessment of: (a) personality (b) delirium, depression, hearing, dementia (c) family conflict (d) medication refusal only

10. Extended-family caregiving should be assessed by asking: (a) whether the mother is attuned (b) who the child seeks when frightened (c) the parents' marital status (d) the family's income

Answer key

1 c · 2 b · 3 b · 4 b · 5 b · 6 c · 7 b · 8 c · 9 b · 10 b

Short answer

1. Distinguish delay from regression, with one clinical example of each.
2. Name three developmental tasks of adolescence and the corresponding family tasks.
3. Why is "let him cry it out" a relational intervention, not only a behavioural one?
4. Give two cultural critiques of attachment theory and one way to adapt your assessment.
5. What are the four parts of effective parent feedback?

Essay (choose one, 1,800 words)

1. "Normal development is a cultural achievement, not a biological fact." Discuss with reference to attachment and independence.
2. Using a disguised case, show how a developmental formulation changed your understanding of a presenting problem that had been framed as behavioural.
3. Critically evaluate the evidence that couple satisfaction declines after the birth of a child, and discuss the clinical implications.

Further reading

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Next: MFT 704 — Ethical and Legal Issues in Therapy